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POLICY MEMORANDUM #5001

EFFECTIVE DATE: 01/26/94

APPROVED BY:

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SUBJECT: EMERGENCY MEDICAL DISPATCH OPERATION REQUIREMENTS

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220; Title 22, Chapter 4, Article 5, 100167.

PURPOSE/POLICY:

To establish guidelines and minimum community standards for organizations and/or ambulance service providers who are directly responsible for the dispatch of ambulances to medical emergencies.

- I. **CONTRACT** – Only those agencies having a Contract/Memorandum of Agreement (MOA) with Solano County shall engage in provision of Emergency Medical Dispatch (EMD) services.
 - A. By December 31, 1994 (or an earlier date for public and private organizations electing to provide Emergency Medical Dispatcher services if approved by the Health Officer) all ambulance provider agencies, Public Safety Answering Points (PSAPs) or other agencies responsible for direct emergency medical dispatch of an ambulance shall have a contract/MOA with Solano County and be authorized by the County Health Officer to provide Emergency Medical Dispatcher (EMD) services utilizing approved EMD Protocols. The EMS Office shall enter into a MOA/Contract with any private or public organization electing to provide ambulance dispatch services within Solano County after first approving its EMD dispatch procedures and ambulance system status management operational plan. The MOA for direct emergency medical dispatch may or may not be part of the mandatory EMT-P Service Agreement required by Section 100167 (b)(4).

- B. Any Public or Private Organization that directly controls and dispatch ambulances to pre-hospital medical emergencies are required to submit their operational plan for ambulance dispatch and its ambulance system status management plan for medical (emergencies) approved before a MOA or Agreement is submitted to the Board. The operational plan at a minimum shall include:
1. An organizational system status management plan that will ensure optimal ambulance coverage for the entire geographical area being serviced (A group plan can be submitted if more than one ambulance service shares a geographical area).
 2. The organization's medical call prioritization protocols and pre-arrival medical instructions.
 3. The organization's quality assurance plan to monitor dispatch centers' performance and compliance with medical community standards.
- C. Public Safety Answering Points (PSAPs) who do not directly dispatch ambulances are not required to have an MOA for EMD (Emergency Medical Dispatcher) services being provided. If a PSAP elects to implement a First Responder Call Prioritization Dispatch System or initiate a program of pre-arrival medical instructions within their service area, an MOA with the EMS Agency is needed prior to implementation of any EMD services.
- D. Dispatch Agency shall be classified – Emergency Medical Dispatch Centers in Solano County directly responsible for dispatch of ambulance for emergency response shall be classified as one of the following:
1. CMED Center (Central Medical Emergency Dispatch) Center – The CMED shall assume the prime county responsibility for determination of the closest appropriate ambulance for emergency calls (excluding Vacaville and areas of Military Bases that deny civilian ambulance access to emergency calls), maintain the EMS Agency-approved system Status Management Plan, and insure that ambulances are always available to immediately respond to medical emergencies county-wide. The CMED shall also coordinate ambulance response for MCIs, medical disasters, coordination of medical response and/or evacuation for mutual aid assistance outside the County or in conjunction with the military. The CMED, when operational, shall also be the designated dispatch center for EMS aircraft for Solano County.
 2. EMAD (Emergency Medical Ambulance Dispatch) Center – An EMAD is an authorized dispatch center that shall assume the responsibility of ambulance dispatch in an area or zone where the community elects not to have the CMED assume responsibility for ambulance dispatch (e.g., the Vacaville PSAP). An EMAD shall provide the EMD services of pre-arrival instructions and call prioritization. Additionally the EMAD shall maintain a written agreement and a Medical Mutual air Plan that meets the following need:

- a. A Mutual Aid Plan which allows both ambulance dispatch centers (the CMED & the EMAD) to have immediate access to available ambulance if either center is attempting to manage a medical emergency where there is an attempt to manage an unusual system overload or an unplanned high demand for ambulance response.
- b. A plan that will insure that each respective ambulance dispatch center will have an immediate inventory of all available ambulances within its zone within 60 seconds upon request.
- c. A plan to convert all ambulances under the control of the EMAD to the control of the CMED in the event a County medical disaster by the Health Officer of EMS Medical Director.
- d. A plan to insure that either the CMED or the EMAD shall not hold an emergency priority 1, 2, 3 or 4 call for over 90 seconds and shall initiate an immediate medical mutual aid response with CMED or EMAD respectively.
- e. Any emergency call received at the CMED or the EMAD in which a priority 1, 2, 3 or 4 call is outside the geographic area of the respective CMED or EMD, the ambulance dispatch center shall immediately refer the call to the appropriate CMED or EMAD within 30 seconds upon receipt of the call.

II. THE MEDICAL PROVISION OF EMD SERVICES includes, but is not limited to:

- A. Medical caller interrogation
- B. Medical pre-arrival and treatment
- C. Medical prioritization of ambulance and advise the primary PSAP of the need to initiate a First Responder Response when clinically indicated.

III. MEDICAL PERFORMANCE STANDARDS FOR EMERGENCY MEDICAL DISPATCHERS OPERATING IN A CMED AND/OR EMAD

- A. Only authorized EMDs shall provide pre-arrival medical instructions to clients requesting emergency medical services.
- B. 95% of all EMS emergency calls shall be answered by the dispatch center before the second ring of the telephone.
- C. The maximum acceptable time interval from when the dispatch center receives a request for emergency ambulance services until the ambulance unit is actually enroute shall not exceed two (2) minutes for a priority 1, 2, 3 or 4 call.
- D. Once an EMD determines a patient is breathing and conscious, 30 seconds is the maximum time needed to complete necessary "key questions" on a selected operative medical protocol to determine the appropriate EMS medical response.

- E. The maximum time interval the CMED or EMAD shall place a caller on hold and then dispatch appropriate EMS responding unit(s) shall not exceed 20 seconds.
- F. EMDs shall only provide medical pre-arrival instructions using designated protocols approved by the Health Officer. Any other requests for medical instruction or advice shall immediately be referred to a base hospital.
- G. The maximum time interval an EMD may hold/handle an emergency call before it alerts and dispatches a responding unit shall not exceed 90/60 seconds.
- H. The maximum time interval a primary Public Safety Answering Point (PSAP) may delay the transfer of an emergency request for ambulance services shall not exceed 60 seconds before it transfers the medical call to the CMED or EMAD.

IV. ALL EMAD AND CMED PROVIDERS SHALL HAVE A QUALITY ASSURANCE COORDINATOR who is a physician, registered nurse, paramedic or certified Emergency Medical Dispatcher (EMD) whose duties will include, but not be limited to:

- A. Review identified systems problems (to be mutually determined by the EMS Agency and Medical Dispatch Staff).
- B. Maintain a daily log for the purpose of monitoring hospital diversion status, ambulance performance failure and unusual incidents.
- C. Maintain a daily log of ambulance availability.
- D. Perform confidential conferences, counseling and initiate remedial action when indicated.
- E. Review and analyze Management Information System (MIS) reports relevant to medical dispatch generated by the CMED MIS.
- F. Provide Quality Assurance (Q.A.) monitoring relevant to include documentation of compliance with pre-arrival instructions and call prioritization standards.
- G. Perform tape reviews and audits on a monthly basis to review:
 - 1. Appropriateness of triage.
 - a. Call prioritization
 - b. Medical classification.
 - 2. Appropriateness of pre-arrival instructions.
 - 3. Appropriateness of utilization of EMS providers.
 - 4. Other quality improvement issues (to be mutually determined by the EMS Agency, CMED or EMAD staff & ambulance service providers).
- H. Ensure Consistency – Ensure that all EMD services and the operational elements the CMED's and/or EMAD's operations are consistent with the County's Q. A. plan for ambulance dispatch and medical expectation of the community.

V. (CMED) CENTER and/or EMAD CENTER RESPONSIBILITIES

- A. Dispatching Procedures – Each Solano County Ambulance Service permitted to operate within the County shall equip their ambulance units to be directly dispatched by the CMED or an approved EMAD. Ambulance Service Provider(s), at their expense, shall implement all necessary electronic communications devices prescribed by the EMS Agency for communication with the CMED Dispatcher to facilitate rapid dispatch of Priority 1, 2, 3 or 4 calls. All ambulances permitted by the County shall be equipped with appropriate radios at Ambulance Service Provider's expense.
- B. All Ambulance Service Providers permitted to operate in the County shall participate with the approved County's CMED.
- C. Authorized direction of Units – CMED and/or EMADs shall directly control and dispatch the ambulances dedicated to emergency response according to its EMS Agency approved System Status Plan. When an ambulance is performing medical mutual aid outside its normal area of activity in the jurisdiction of another EMAD or CMED, the unit shall remain under its exclusive direction until released by the requesting CMED or EMAD.
- D. Immediate notification of status – In all cases the Ambulance Service Provider shall immediately notify the CMED Center and/or its appropriate EMAD Center of any change of ambulance availability status, or of any change in communication status in the event there is an unexpected demand for emergency ambulance services that exceed its system status plan or if there is a communication system failure in which the EMAD or CMED cannot function optimally.
- E. Daily radio checks – Ambulance Service Providers shall conduct daily radio equipment checks with both the hospital(s) and appropriate dispatch center(s) at the time and manner agreed upon with those agencies and the EMS Agency.
- F. Any question for immediate ambulance transport (as defined in EMS policy 5520) is assumed to be an emergency request for ambulance services. Ambulance Service Providers shall refer all private emergency calls and/or non-911 non-scheduled requests for ambulance transport to the CMED, or approved EMAD, who will in turn dispatch the closest appropriate ambulance to the call, for any call determine whether the call is a Priority 1, 2, 3 or 4 emergency response. Calls determined not to be a Priority 1, 2, 3 or 4 emergency response shall be managed based on existing transfer agreements or preferential requests from the calling parties requesting non-scheduled ambulance transport.
- G. Stand-by Locations – Ambulance Service Providers shall, if requested, respond an ambulance unit to stand-by locations as specified by CMED. Once an ambulance has been dispatched to a stand-by or posting location, the Ambulance Service Provider may not divert said ambulance for a non-emergency call until released by the CMED. An ambulance committed to stand-by or posting location shall be utilized only for

emergency calls until such time as the CMED or approved EMAD Center authorizes return to its usual base of operations.

- H. Unique number – CMED or EMADs shall issue a unique Authorization Case number to each CODE 3, CODE 2 or CODE 1 call. Each Patient Care Report (PCR) Form generated by ambulance personnel responding to an emergency call shall include the CMED or EMAD case number on the medical record.
- I. Daily submission of log to EMS – The CMED or EMADs shall maintain and provide daily, to the EMS Office; and each ambulance provider service is responsible for an accurate daily dispatch log of all emergency ambulance activities until such time as the CAD and mobile data terminals are installed, debugged and operational to a high degree of reliability when the data can be transmitted electronically. Each Ambulance Service Provider shall notify the EMS in writing, of any and/or each modification to the daily log. The EMS Office notification shall include original and modified entry information and reason for modification until such time as electronic transfer of dispatch data becomes a reality. For each emergency call that exceeds performance standards defined the EMS Policy 5520, the CMED and EMADs shall notify both the EMS Office and the appropriate ambulance provider within 24 hours of a call that requires follow-up and investigation to determine why the call exceeded community performance standards.
- J. CMED and/or EMAD Procedures for Emergency Calls.
 - 1. Issuance of Number – Upon receipt of an emergency request for response, the CMED or EMAD shall issue a unique CMED case number and dispatch the closest appropriate ambulance to the emergency; each Ambulance Service Provider shall immediately respond and as soon as underway, the ambulance unit shall report by radio to CMED or EMAD indicating that it is underway; but under no circumstances shall this report be more than three minutes (180 seconds) from notification to the Ambulance Service Provider of the call. If there is delay of more than 180 seconds and the CMED or EMAD does not know if an ambulance is enroute, the EMAD or CMED shall assign the case to the next closest ambulance unit and initiate a search for the first unit it cannot account for.
 - 2. Response needs prior approval – Under no circumstances shall Ambulance Service Provider respond to any request for emergency service originating in another response area of responsibility without the specific prior approval of CMED or EMAD Center.
 - 3. CMED or EMADs shall assign ambulance to cross response area lines when deemed necessary for the most efficient response to an emergency call in areas serviced by another CMED or EMAD Center.
 - 4. Notification of arrival and departure – Upon arrival at the location to which the ambulance has been dispatched, ambulance personnel shall notify CMED Center or approved EMAD Center; and notification is again required when departing the location with a patient to the hospital.

5. Notification of Hospital – While at the emergency scene or immediately upon departing, ambulance personnel shall notify the Base or receiving hospital as appropriate via MEDNET/Med Channel Radio, or other approved communication system, of the anticipated delivery of patient(s), a description of the possible injury(ies) or illness, appropriate vital signs and the expected time of arrival at the hospital emergency department.
 6. Notification of arrival to CMED – Upon arrival at the hospital, CMED or EMAD will be notified. Ambulance personnel shall obtain the CMED case number along with their call times when completing their medical records. Ambulance personnel shall also provide their PCR(s) case numbers to the CMED or EMAD to be included in the computer CAD system file for the case.
 7. Notification of availability – upon becoming again available for service, CMED or EMAD will be notified. The CMED or EMAD shall determine the availability of an ambulance unit within 12 minutes after arrival if the transporting crew does not confirm its availability after arriving at a medical facility and turning the patient over to the medical staff.
- K. The CMED Center shall maintain records of all times listed above for each call, and the assigned unique identification number for each dispatched ambulance unit. CMED shall also maintain a record of the Patient Care Report number on each patient transported, each dry run, each case where the patient was found dead, each request for stand-by and each emergency Interfacility transfer.
- L. CMED and/or EMADs shall routinely provide the following reports to Ambulance Service Provider and to EMS Agency:
1. Monthly. Summary Activity Reports using a format available from the CAD system of the CMED or EMAD that is mutually agreed upon between the dispatch center management and the Health Officer (or his/her designee).
 2. Monthly. The necessary management reports obtained from the CAD system's management information system to allow the EMS office staff to analyze and evaluate the performance of each ambulance service's compliance rate with the medical performance standards. The reports generated at a minimum shall allow for qualitative measurement of the following performance criteria:
 - a. Each ambulance service shall provide 100% of coverage of all requests for ambulance service within its contracted service area and each ambulance response adhered-to response time standards.
 - b. A listing of each Dispatch/Response Delays.
 3. Daily: Download of dispatch records and required data elements for each emergency call the Center received requiring dispatch of an ambulance.

4. Daily: An “unable to Dispatch” Unusual Occurrence Form. During any period of time a situation results when there are insufficient ambulance units available for service, the CMED or EMAD will make reasonable efforts to obtain back-up services from adjacent areas to provide coverage for ambulance service provider(s)’ area. In each instance this situation occurs, the CMED or EMAD shall notify the EMS office which [will require], at a minimum, an unusual occurrence report including the following information:
 - a. The date, length of time in seconds, the request for emergency ambulance services was unanswered;
 - b. The status of all other ambulance units in the zone at the time of request for service;
 - c. The reason the call could not be answered; EMS staff shall investigate each reported incident after receipt of such information.
5. Daily; a listing of all performance failure occurrences/unusual occurrence reports regarding any Emergency Call Priority 1, 2, 3 or 4 where:
 - a. CODE 3 calls which fall outside the performance standards for urban, rural or remote.
 - b. Ambulance “out of the chute times” of more than two minutes for Priority 1, 2, 3 or 4 emergency calls;
 - c. The requesting party called back requesting the status of the responding ambulance after 5 minutes from emergency call;
 - d. Any call where vehicle malfunction or accident resulted in a delay in a response.
 - e. Any non-scheduled request for an immediate ambulance response (as defined in EMS Policy 5520) in which the ambulance service provider elects to refer the call to another ambulance service.

It is the responsibility of the CMED or EMAD to notify the Ambulance Service Provider and the EMS Office of this type of situation. It is the responsibility of the Ambulance Service Provider to investigate the unusual occurrence and report the following information to the EMS Office within 72 hours, or three (3) working days after being notified of a performance failure. The ambulance service provider report shall contain the following results of its investigation:

- (1) Reason(s) for performance failure, vehicle failure and/or malfunction;
- (2) Effect of vehicle or performance failure upon patient clinical condition;

- (3) Actions Ambulance Service Provider has taken to prevent other such failures;
 - (4) Dual PCRs – In each instance where patient transport is delayed resulting from a vehicle failure and/or malfunction, the ambulance service provider will assure that EMS personnel on the vehicle that failed and the EMS personnel on the vehicle who transport the patient will submit distinct, separate prehospital report forms regarding the medical care the patient received from each unit.
 - 6. Monthly; an EMS Aircraft Dispatch Utilization Report (CMED only).
 - 7. Summary Report for MCI/Disasters – Within 24 hours or one working day of each MCI or medical disaster occurrence, a summary report of the activities that occurred at the CMED and/or EMAD for all cases where:
 - a. Three or more ground EMS units responded to a single incident, or
 - b. Two or more ground units and a single EMS aircraft responded to a single incident.
 - 8. Quarterly 911 Report – Quarterly submission to the EMS Office and thence to the County EMCC, a report of 911 utilization and contract response to 911 calls.
- M. Fiscal, Responsibilities to Maintain CMED – All Air and Ground Ambulance Service Providers shall pay their fair share of their dispatch costs to maintain the designated County CMED to the CMED provider organization.
- N. Dispatch Evaluation – Organizations responsible for ambulance dispatch shall implement an on-going program for the evaluation of dispatch operation, education and training of dispatchers, and ensure problem identification and resolution.
- O. Dispatch Service(s) of the CMED/EMADs
 - 1. Emergency Medical Dispatch Standards
 - a. Policies and Procedures – Dispatch of emergency ambulances shall be according to the policies, procedures and protocols promulgated by the County Health Officer which are developed in collaboration with representatives of the Dispatch Center, EMS Agency and the ambulance service providers. In each instance that an ambulance does not attain the response time standard for an emergency response, the CMED or EMAD dispatcher identify the call and refer it back to the ambulance service operator to complete an “exception report” using a report form approved by the Health Officer. The CMED or EMAD dispatcher shall concurrently notify the EMS office of the occurrence.
 - b. Training and Certification – Each CMED and/or EMAD shall employ Emergency Medical Dispatchers who have successfully completed an Emergency Medical Dispatch Program according to the standards

specified by the EMS Medical Director. All Emergency Medical Dispatchers shall be certified as Emergency Medical Dispatchers by the EMS Medical Director by 12/31/94. The interim period shall be designated a transition period for EMDs to obtain certification.

c. Continuing Education Recommendations for EMDs.

- (1) Each CMED or EMAD shall organize opportunities for education including maintenance and proficiency of EMD skills as specified in this policy.
- (2) Each CMED or EMAD shall maintain an ongoing program of continuing dispatcher education. Continuing dispatch education, at a minimum, shall include a review of elements of the EMD curriculum, with special emphasis on operational functions, protocols, policy compliance, new procedures, medical advancements, problematic situations and a greater in-depth understanding of the medical conditions that are associated with the emergency Medical Dispatch Priority Reference System.
- (3) The EMD Dispatch Center shall train and update dispatch personnel regarding MCI/Disaster radio communications and disaster response plans.
- (4) Training should be consistent with EMS Agency EMD Certification and re-certification requirements.

2. Emergency Medical Dispatcher Staffing – The CMED and/or EMADs staffing shall be configured that an EMD shall be capable of immediately dispatching the closed appropriate ambulance, maintain its System Status Plan, and provide pre-arrival instruction 24 hours/day 7 days/week; and 95% of the emergency calls on or before the second telephone ring without any delay.
3. Control of Ambulances – CMED and/or EMAD dispatchers shall follow the System Status Plan provided by the Ambulance Service Provider(s) for the deployment and dispatch of emergency ambulances within its area of ambulance dispatch responsibility.
4. Dispatch Quality Assurance Program – There shall be a comprehensive quality assurance program for emergency medical dispatch performance. This program will be developed jointly by the Ambulance Service Providers, EMS Agency, the organization responsible for actual dispatch and the County Communications Division (if applicable). The program will include, but not be limited to, evaluation of adherence to the EMD Policies and Protocols and System Status Plan. Evaluation methodologies will include, but not be limited to, data collected from the CAD System and tape recordings of voice communications and retrospective audit. Tape recordings and retrospective audits shall be accessible to Ambulance Service Provider(s) and the EMS Agency at the Dispatch Center, on an appointment basis or upon request. M

Copies of tape recordings shall be made available to the Ambulance Service Provider, and the EMS Agency. Summary results of retrospective audits shall be submitted to the EMS Office at the end of each month. Q. A.

responsibilities of the CMED and/or EMAD management shall include:

- a. Maintaining appropriate Q. A. personnel to daily review ambulance activation, dispatch and response time intervals and investigate all ambulance performance failures, and submit findings to the EMS Office within 72b hours of each case that is identified as beyond the threshold of performance expectations.
5. Access to CMED and EMADS – EMS Staff shall have access to the CMED or approved EMAD Center upon reasonable notice. The declaration of a medical disaster, MCI incident or periods of time when the System Status Plan is not in effect due to system overload are times when EMS Staff shall have immediate access to the CMED. Additionally, Ambulance Service Provider(s) and the EMS Office shall have access to the CAD system via computer terminals for monitoring and data download purposes only. The CMED shall be designed to allow EMS Office Staff SYSOP/Administrator access to the emergency ambulance CAD system to monitor performance of the ambulance services and the performance of the CMED dispatchers.
6. Computer Aided Dispatch – The Dispatch Center shall utilize a computer aided dispatch system (CAD), comprised of computer hardware and software, for the dispatching of Ambulance Service Provider(s)' emergency ambulances. Policies of dispatching the closest appropriate ambulance shall be determined by the CAD, and the County approved System Status Plan and approve CMED/EMAD ambulance dispatch policies.
- P. Equipment Maintenance – CMED/EMAD management will establish and maintain a preventative maintenance program for CMED or EMAD equipment and software.
- Q. Changes in Equipment and Supply Requirements – All proposed changes, updates and revisions to CMED dispatch equipment, software and supply items shall be mutually agreed upon by the management of the CMED/EMAD, Ambulance Service Providers being dispatched, County Communications and the EMS Agency.

VI. THE CMED AND ALL EMADs SHALL ADHERE TO THE STANDARDS FOR COMMUNICATIONS AS APPROVED BY THE HEALTH OFFICER, AND ALL EMERGENCY COMMUNICATIONS STANDARDS FOR HARDWARE AND EQUIPMENT AS PRESCRIBED BY THE DIRECTOR OF THE SOLANO COUNTY COMMUNICATIONS DIVISION (see Attachment A) to include, but not limited to:

- A. Computer hardware and software:
 1. CAD – The CMED shall operate a computer aided dispatch (CAD). (The CMED CAD system shall be developed to be capable of integration with

Mobile Data Terminals (MDTs) and Automatic Vehicle Locator (AVL) system.

2. CAD lock outs and requirements – The CAD system shall lock out all manual attempts to edit or change any CAD case record. The system shall create a duplicate record for any and all changes when there is an attempt to change or edit a record and maintain a separate file for each change in a dispatch record. The file record shall be capable of identifying pre and post changes, the number of times the record was changed, the individual who made the changes and a date stamp when the changes occurred.
 3. The CAD system shall maintain, at a minimum, the following data elements and records in the MIS:
 - a. The PCR or triage tag numbers for each case an emergency response occurred.
 - b. Transfer/referral of calls – the CAD shall retain the following information when a primary PSAP or secondary PSAP transfers and/or refers an emergency call to an EMAD or CMED (the time the PSAP first received the call, the time the PSAP referred the call, to the CMED or EMAD, the referring PSAP's unique case number, in addition to all necessary information the CMED and/or EMAD needs to ensure the closest appropriate ambulance shall respond to the emergency.)
 - c. Additional data elements – the CMED's CAD system shall additionally maintain the following information (data elements) for each emergency call for ambulance response in which the requesting party did not use the 911 system to request immediate ambulance services. The following data elements for each ambulance response shall be maintained in the CAD's MIS: (the requesting party or individual (e.g., Kaiser Triage Nurse, CMF Guard Station, a specific nursing home, etc).) any special request for mode of transport or type of vehicle to respond or any preferential request regarding mode of response (CODE 2 or CODE 3) which would inhibit the dispatch of the closest appropriate ambulance.
 4. CAD Audit Requirements – the CAD system shall be capable of identifying and tracking each case in which the CMED and/or EMAD agency provided pre-arrival instructions and/or call prioritization in its response. The system shall be capable of providing all necessary information to allow a Q. A. auditor to perform a medical retrospective review of any case in question.
- B. Telephone System Linkages.
1. The CMED and EMAD shall have a direct line or radio line to each ambulance service quarters within its direct service area.
 2. The CMED shall have a dedicated linkage with each Primary PSAP in the county. The CMED shall be capable of receiving emergency referrals from

any primary PSAP after the primary PSAP engages a selective routing switch on their ANI/ALI display console(s). The Primary PSAPs are:

- a. Solano County Sheriff's Office Dispatch
 - b. California Highway Patrol
 - c. Benicia Police Department
 - d. Vallejo Police Department
 - e. Fairfield Department of Public Service
 - f. Suisun City Police Department
 - g. Vacaville Police Department
 - h. Travis Air Force Base (ANI/ALI is optional)
 - i. Mare Island Naval Shipyard (ANI/ALI is optional)
3. Each EMAD shall maintain a direct linkage with the CMED to allow for medical mutual aid, medical disaster response, EMS aircraft dispatch and CMED monitoring of ambulance availability within the EMAD's zone of coverage.
 4. Each EMAD and CMED shall maintain a dedicated Civil Defense phone linkage with the Solano County Emergency Operations Center (EOC) and Sheriff's Office Dispatch Center.
 5. Each EMAD and the CMED shall maintain sufficient incoming telephone lines to ensure all requests for emergency ambulance transport are answered by the third ring of a phone, 95% shall be answered on or before the second ring.
 6. Each EMAD and CMED shall be capable of functioning as an on-line medical control transfer point in the event a radio or phone linkage with a base hospital is not immediately accessible to an ALS unit needing immediate base consults.

C. Radio Communications Systems

1. Each EMAD and the CMED shall have the capability of communicating (transmitting and receiving) with all ambulances providing service to Solano County on the following radio communications back-up channels:
 - a. Med-Net (HEAR)
 - b. Med Channels 9 and 10 (dispatch)
2. The CMED shall have the capability of dispatching ambulances within its service area on an appropriate radio system that shall be approved by the Health Officer and the director of County Communications. The dispatch system shall be capable of supporting MDTs and AVL technologies. The CMED may initiate MDT/AVL technology using a "phase in" approach, but this limitation in technology does not release the CMED or ambulance service providers from their responsibility to:

- a. Dispatching the closest ambulance to a medical emergency.
 - b. The responsibility to maintain a System Status Plan, appropriate posting of ambulance units and ensure enough ambulances are available to meet public demand.
 - c. Provide the EMS Office necessary information to monitor performance compliance with medical standards.
3. The CMED shall maintain and operate a MDT/AVL network control processor within its CAD system.
- D. Recording equipment with retention time of tapes.
 1. Tape recordings; CMED – The CMED shall maintain a logging recorder capable of monitoring 8 channels and retain tape recording(s) of all CMED operational activities for no less than 45 days. The system shall have the capability of instant playback with 5 minutes capacity each. The CMED shall be capable of providing tape records of CMED activities to the Ambulance Services it dispatches for and the EMS Agency within 24 hours upon request. The recording capability of the CMED shall be configured that when EMDs or management wish to obtain an instant playback of an event, the recording system shall be configured to do so without losing access to recording current activities in progress.
 2. Tape recordings; EMAD – Each EMAD shall maintain a logging recorder system with instant playback capability. Each EMAD shall be able to provide tape records of its operational activity to the ambulance organization it services, the EMS Agency and QI staff of the CMED within 24 hours upon request.

VII. CMED AND/OR EMAD DEPLOYMENT OF AMBULANCES

All EMADs, the CMED and all Solano County Ambulance Service Providers shall be responsible for planning the dispatch of ambulances through the provision of a system status plan. The actual dispatch of ambulances in some zone(s) shall be by the CMED Center, a joint private/public venture dedicated to providing medical pre-arrival instructions, dispatch of the closest available ambulance, posting of ambulance units, pre-arrival instruction and maintenance of the EMS Agency approved System Status Management Plan.

A. System Status Plan

1. Written Plan – All EMADs, Ambulance Service Providers or CMED, in conjunction with the Local EMS Agency and CMED Dispatch Staff, will be responsible for providing a written System Status Plan for the number of ambulances, their assigned locations and deployment strategies. The plan shall be provided to the EMS Agency, for Health Officer approval, 30 days before the C-MED becomes operational.
2. Ambulance Stations (or Posting Positions) – Units shall be placed in such a manner to permit, during normal traffic conditions, to adhere to the minimum urban response time standard identified in Policy 5520, from when the CMED receives the call until unit arrives on-scene time, to all urban areas within the County. Locations of ambulance units may be either fixed geographical stations or posting positions assigned by CMED as the result of implementation of the EMS Agency approved System Status Management Plan.

B. Posting of Back-up Services

1. When any EMAD Ambulance Service Provider or CMED does not have an ambulance unit available to respond, CMED shall request back-up coverage of an available ambulance from an adjacent response area.
2. The back-up ambulance will be relocated to a pre-designated location according to the EMS Agency approved System Status Management Plan. Such locations will be mutually agreed upon by all Ambulance service Providers in the EMS System and the County Health Officer (or his designee).
3. All ambulance services in the County shall agree to respond to a request from an authorized CMED to relocate any available ambulance to the pre-designated back-up location.
4. CMED will dispatch the back-up ambulance to any immediate response call in the vacant response area and immediately notify the responsible ambulance service provider management providing back-up service.
5. Ambulance Service Providers providing back-up services outside its area of responsibility may bill the responsible ambulance service for stand-by time at a rate approved by the Board.

- C. Posting of Ambulances during periods of Public Health Emergencies – Ambulance Service Provider agrees, to the best of the Ambulance Service Provider's ability, to assist in servicing any other response areas for which the Emergency Ambulance Service Agreement has been suspended or terminated upon request of County Health Officer. This period shall be declared an Unusual System Overload and response time Standards and Penalties are temporarily suspended until the emergency is resolved.

VIII. STANDARDS FOR COMMUNICATION EQUIPMENT IN AMBULANCES

- A. Requirements & Specifications – Ambulance Service Providers shall provide all communications equipment as required and specified by the County Health Officer in coordination with the County Communications Division for each ambulance owned and operated in Solano County. All ambulances performing emergency response in this County shall have, at a minimum:
1. One mobile radio with the capability for communications with the CMED, Base Hospital and its designated EMAD if indicated on the primary radio system. (Technical specifications will be provided as an attachment to this policy when the CMED is operational).
 2. One portable 5 watt radio, for communications on the primary radio system with the CMED or EMAD from remote locations away from the ambulance. (Technical specifications will be provided as an attachment to this policy when the CMED is operational).
 3. One mobile radio with the capability to communicate on all ten MED channels with the CMED, EMAD, Base Hospitals and receiving hospitals. MED Channels 9 & 10 to be used for declared MCI and/or medical disaster situations, ambulance communications between CMED or EMAD's may occur on MED Channel 9 or 10 leaving MED-NET available for inter-hospital communications and the MCI Field Supervisor and in controlling base hospital consultations (technical specifications will be provided as an attachment to this policy when the CMED is operational).
 4. One 5-watt portable radio for communications on all ten MED channels from remote locations away from the ambulance. This Bio-telemetry capability on the MED channels is optional. A vehicular repeater for use with the portable radio may be needed on an individual provider basis. Technical specifications will be provided as an attachment to this policy when the CMED is operational.
 5. One mobile MED-NET radio for situations requiring medical mutual aid, ambulances should coordinate with the CMED or EMAD's on MED-NET. Technical specifications will be provided as an attachment to this policy when CMED is operational.
 6. A transportable 3-watt cellular phone is recommended but not mandated.

7. Certificate of inspection for radio equipment. Ambulance Service Providers shall obtain a Certificate of Inspection of approved radio equipment following installation in each ambulance and on an annual basis thereafter. These vehicles shall be made available to the EMS Agency upon reasonable request. Alternate procedure for radio inspection may be adopted by the County Communications Division. The cost for certificate of inspection shall be obtained from individual ambulance permit fees that are required annually by the EMS office. Public ambulance services exempted from paying ambulance permit fees shall pay all County Costs for the inspection of radio equipment.
8. Approval of Radio Equipment – All radio equipment used for EMS operations other than the ambulance service provider's internal company system must be approved by the County Communications Manager.
9. Medical Needs – Ambulance Service Providers shall obtain, install and maintain all such radio equipment as is deemed by Health Officer to be appropriate for the transmission of voice communications, biotelemetry, data and AVL data for medical direction by paramedic base hospitals designated to operate in the Solano County EMS System.

**IX. OTHER NON-AMBULANCE SERVICES PROVIDED BY SOLANO COUNTY
AMBULANCE SERVICE PROVIDERS**

- A. Non-emergency ambulance or Fire Response Services – Ambulance Service Provider(s) shall not be prevented from conducting or providing private, non-emergency ambulance services or fire response services, provided that such business or other public safety activity does not in any way interfere with the requirements, duties and obligations to ensure an appropriate ambulance response and availability to all residents of Solano County.
- B. Dedicated paramedic ambulances may not be used for scheduled non-emergency Interfacility transfer if those units.... Unless allowed under the System Status Plan developed by the Ambulance Service Providers.
- C. Responsibility to redirect – CMED and/or EMAD Dispatch Staff have the responsibility to re-direct a non-dedicated Paramedic Ambulance to respond to an emergency if the unit dispatched to an Interfacility transfer has not yet boarded a patient for transport according to the System Status Plan.
- D. Back-up for ALS electing non-transport – Only the CMED or an EMAD with medically approved call prioritization procedures may routinely dispatch a (BLS) Ambulance to an emergency in which an on-scene paramedic unit has previously arrive on-scene and elected not to transport a patient needing transport services.

SUBJECT: Emergency Medical Dispatch
Operation Requirements

Policy: 5001
DATE: 01/26/94

ATTACHMENT A

The Solano County Communications Division is still in the process of developing CMED Standards.

The Solano County Communications division should be advised of any planned purchase prior to purchase of any radio or landline communications equipment.



Health & Social Services Department

Donald R. Rowe, Director

Public Health - Mental Health - Public Guardian
Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

Public Health Division
Thomas L. Charron, M.D., M.P.H.
Health Officer
Deputy Director Health and Social Services

EMERGENCY MEDICAL SERVICES
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EMERGENCY MEDICAL SERVICES

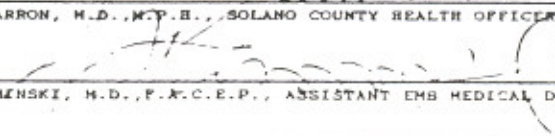
POLICY MEMORANDUM #5100

EFFECTIVE DATE: 11/01/92

APPROVED BY:


THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:


STEVEN J. KAMENSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: AUTHORITY FOR MEDICAL EMERGENCY SCENE MANAGEMENT

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1798.6, 1797.90,
1797.94, 1797.182, 1797.188, 1797.204, 1797.220

PURPOSE/POLICY:

To define the authority for medical emergency scene management.

I. AUTHORITY FOR PATIENT HEALTH CARE MANAGEMENT

Authority for patient health care management in an emergency shall be vested in such licensed or certified health care professionals, which may include any paramedic or other prehospital emergency personnel at the scene of the emergency, who is the most medically qualified specific to the provision of rendering emergency medical care. If no licensed or certified health care professional is available, the authority shall be vested in the most appropriate medically qualified representative of public safety agencies who may have responded to the scene of an emergency.

II. MANAGEMENT OF THE SCENE

Notwithstanding the above, authority for the management of the scene of an emergency shall be vested in the appropriate public safety agency having primary investigative authority. The scene of an emergency shall be managed in a manner designed to minimize the risk of death or health impairment to the patient and to other persons who may be exposed to the risks as a result of the emergency condition, and priority shall be placed upon the interests of those persons exposed to the more serious and immediate risks to life and health. Public safety officials shall consult with emergency medical services personnel or other authoritative health care professionals at the scene in the determination of relevant risks (Health and Safety Code 1798.6).

III. EMERGENCY CRIME SCENE ENTRANCE

Emergency personnel will not enter an emergency crime scene until appropriate public safety personnel have arrived and secured the scene to make entry reasonably safe.

IV. MEDICAL MANAGEMENT AT THE SCENE

Medical management at the scene of a medical emergency includes:

- A. Medical evaluation
- B. Medical aspects of extrication and all movement of the patient(s).
- C. Medical care
- D. Patient destination
- E. Transport code
- F. Medical consultation with a Base Hospital.

V. ORDER OF RESPONSIBILITY

Responsibility for emergency medical management is based on the following order as personnel arrive on the scene:

- A. Law enforcement personnel
- B. First Responder (First Aid Certified)
- C. First Responder EMT-1
- D. Transport Unit EMT-1
- E. Non-transporting paramedic EMT-P or Transport Unit EMT-P that arrives on-scene first.



Health & Social Services Department

Donald R. Rowe, Director

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Adult and Child Services - Eligibility & Employment Services
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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #5200

EFFECTIVE DATE: 11/01/92

APPROVED BY:

Handwritten signature of Thomas L. Charron.
THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

Handwritten signature of Steven J. Ryminski.
STEVEN J. RYMINSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: EMS FIRST RESPONDER DEFINITION

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.90, 1797.94

PURPOSE/POLICY: To define a Solano County First Responder Agency.

I. FIRST RESPONDER AGENCY IS:

A first responder agency is any public and/or private agency that, within its geographic jurisdiction or service area, elects to respond to medical emergencies concurrently with an EMS ambulance Service provider(s). At a minimum, a first responder agency's prime function when responding to the scene of a medical emergency is to provide emergency first aid and basic life support until arrival of an ambulance.

II. FIRE SUPPRESSION AGENCIES AS FIRST RESPONDERS

In Solano County, law and/or fire suppression agencies electing to respond to 911 requests for medical emergencies at a minimum are designated as a public safety first responder agency within their respective jurisdiction.

III. MINIMUM STANDARD OF MEDICAL TRAINING

The minimum standard of medical training for first responder personnel is First Aid for Public Safety Personnel as defined by Chapter 1.5 or Title XXII of the California Code of Regulations.

IV. POLICIES FOR 911 MEDICAL EMERGENCY DISPATCH

First responder fire agencies electing to provide medical first responder services shall, in conjunction with law enforcement agencies within their respective jurisdictions, establish policies to insure that first responders are appropriately dispatched to all 911 requests for medical emergencies. The only exceptions shall be those agencies where a call prioritization system, approved in writing by Solano County EMS, prioritizes a 911 request as not needing immediate EMS intervention and not needing the concurrent dispatch of a medical first responder agency.

V. THREE CLASSIFICATIONS OF FIRST RESPONDER AGENCIES

The Solano EMS System recognizes three classifications of First Responder Agencies:

- A. Public Safety First Aid – Most public safety fire and police agencies electing to respond to 911 requests for medical emergencies are automatically classified as Public Safety First Responders. Public Safety Agencies are required to have all personnel, at a minimum, trained to standards as defined by Chapter 1.5 of Title 22 of the California Code of Regulations. Medical reporting requirements are limited to records prescribed by the Public Safety Agency and to medical first aid equipment routinely stocked by the Public Safety Agency. Public Safety agencies electing to routinely respond to 911 requests for medical emergencies should comply with Quality Assurance standards prescribed by the EMS System Quality Assurance Plan (Policy #2205). It is recommended that all Public Safety First Aid Agencies have a written agreement with the EMS Office that defines the level of EMS service they shall provide to their community. Public Safety First Aid Agencies may provide defibrillation services in their service area if they adhere to the standards of Policy #3300.
- B. Public Safety EMT-1 = Public Safety Agencies electing to provide EMS services at a higher level than the minimum first aid standards shall be classified as a Public Safety EMT-I Agency and shall adhere to the following requirements:
 - 1. Contract/MOA – All Public Safety EMT-I Agencies shall have a contract/MOA with Solano County and be authorized by the County Health Officer to provide emergency medical services at the MET-I or EMT-D level. The public safety EMT-I agency may elect to bill for medical services rendered, but all charges for medical services shall be included in their Memorandum of Agreement (MOA) and approved by the Board of Supervisors.
 - 2. Quality Assurance Coordinator – All Public Safety EMT-I Agencies shall have a quality assurance coordinator who is a physician, registered nurse, paramedic or an authorized EMT-I.
 - 3. Medical Control – All Public Safety EMT-I Agencies shall comply with all medical control requirements including medical records, dispatch, stocking of equipment and supplies, patient care guidelines and quality assurance requirements prescribed by the EMS System Quality Assurance Plan (Policy #2205).
 - 4. Defibrillation – All Public Safety EMT-I Agencies shall provide emergency defibrillation using automatic or semi-automatic defibrillators unless exempted in writing by the EMS Medical Director.

- C. Public Safety EMT-P Agency – All Public Safety Agencies wishing to be classified as an EMT-P Service Provider shall adhere to the following requirements:
1. A fire department or other private/public safety organization may be approved to provide EMT-P service as a first responder agency in the County EMS System providing that such paramedic service is provided in a service area which has paramedic emergency ambulance service approved by and operating in conjunction with the County EMS System.
 2. The provision of EMT-P service by an approved first responder unit shall be predicated upon the concurrent dispatch of an ALS transport ambulance staffed with EMT-P personnel.
 3. The first responder paramedic unit shall be staffed with one (1) or more certified and accredited EMT-Ps. The first arriving EMT-P has full responsibility for the patient until that patient is formally turned over to an equal or higher trained person. EMT-Ps from the first responder paramedic unit should accompany the patient to the hospital and assist the transport unit EMT-Ps when requested.
 4. If Base Hospital contact has been made, the first responder EMT-P will inform the Base Hospital of the arrival of and transfer of patient care to the transporting EMT-P unit.
 5. EMT-Ps staffing the first responder paramedic unit shall comply with all pertinent treatment protocols and procedures.
 6. EMT-Ps staffing the first responder paramedic unit shall comply with all requirements established by the certifying authority for EMT-P certification/recertification and local accreditation.
 7. The Public Safety EMT-P Agency shall participate in the quality assurance and incident review process established by the EMS Office.
 8. The Public Safety EMT-P Agency shall abide by the following medical management responsibilities.
 - a. Initial responsibility for patient medical management shall rest with the EMT-P member of the first arriving paramedic unit.
 - b. Patient care may be transferred from first responder EMT-P to the transporting EMT-P unit after a verbal or written patient report is given. A copy of the first responder PCR written report shall be given to the transporting unit or the non-transporting EMT-P shall insure that the report will be submitted to the receiving hospital not later than 1 hour after the non-transporting EMT-P unit clears the scene.
 - c. The First Responder EMT-P should assist the transporting EMT-P on scene and/or enroute to the hospital, as requested, and as determined appropriate by the EMT-P caring for the patient.
 - d. Transporting EMT-Ps shall have all necessary equipment available to manage the patient at the time of transfer. Documentation of the transfer shall be recorded on the PCRs by both first responder and transporting EMT-Ps.

- e. Both first responder and transport EMT-Ps shall initiate a PCR for each patient contact. The first responder agency will ensure that receiving/base hospitals receive PCRs according to EMS policy.
- 9. A public safety EMT-P agency may elect to bill for medical services rendered but all charges for medical services shall be included in the MOA and approved by the Board of Supervisors.
- 10. All Public Safety EMT-P Agencies shall have a contract/MOA with Solano County and be authorized by the County Health Officer to provide emergency medical services at the EMT-P level.
- 11. All Public Safety EMT-P Agencies shall have a quality assurance coordinator who is a physician, registered nurse or paramedic.
- 12. All Public Safety EMT-P Agencies shall comply with all medical control requirements including medical records, dispatch, stocking of equipment and supplies, patient care guidelines and quality assurance requirements prescribed by the EMS System @Quality Assurance Plan (Policy #2203).

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Patrick O. Duterte, Director

Steven P. Whiteley, MD
EMS Agency Medical Director

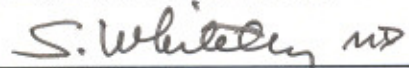
EMERGENCY MEDICAL SERVICES AGENCY
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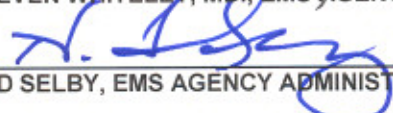
Ted Selby
EMS Agency Administrator

POLICY MEMORANDUM 5300

Implementation Date: February 15, 2012
Release Date: Dec 30, 2011

REVIEWED/APPROVED BY:


STEVEN WHITELEY, MD, EMS AGENCY MEDICAL DIRECTOR


TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: ROLE OF EMS FIRST RESPONDER

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.90, 1797.94, 1797.182, 1797.183, 1797.176, 1797.204, 1797.220, California Government Code section 3100,

PURPOSE: To define the role of Emergency Medical Services (EMS) First Responders.

I. DUTIES OF EMS FIRST RESPONDERS

First Responders shall perform the following duties when responding to requests for emergency medical services.

- A. Ascertain the nature of the problem. This should include a determination of:
 1. Level of distress: life-threatening (emergent), urgent, extrication needed, other medical resources, need for EMS Aircraft, etc.
 2. Evaluation of Airway, Breathing, and Circulation (ABC's), level of consciousness, vital signs, skin signs, etc.
- B. Institute immediate first aid as indicated and other Basic Life Support (BLS) emergency care as appropriate (e.g., oxygen administration, CPR, defibrillation, cervical spine immobilization, etc.)

- C. EMS Agency personnel may act as first responders in certain circumstances and are designated as Disaster Services Workers, per California Government Code. In this role they may need to respond and perform functions based on their EMS knowledge, medical training, and licensing.
 - EMS Agency personnel may use an emergency response vehicle. EMS Agency personnel using the emergency response vehicle for official business will complete an Emergency Vehicle Operations Course (EVOC).

II. MEDICAL MANAGEMENT at the scene of a medical emergency should be handled in the following manner: Responsibility – Upon arrival of the first responder, one person, usually the most medically qualified (subject to departmental policy), assumes responsibility for care of the patient. Turnover of responsibility – The first responder continues responsibility for patient care until arrival of an Advanced Life Support (ALS) agency, at which time first responder provides a verbal or written report to ALS personnel. This report should include patient assessment, findings, and emergency care rendered prior to arrival of the ALS agency. This policy does not supersede Policy 6140 Responsibility for/Transfer of Patient Care.

- A. Acceptance of responsibility – ALS Fire Agency or ALS transport ambulance unit shall assume responsibility for patient care after receiving a verbal or written report from the first responder. If written reports of first responder care are not made available to transporting agency, it is the responsibility of the first responder agency to ensure medical records of the first responder care are made available to the hospital receiving the patient.
- B. The first responder unit should routinely remain on the scene until the transport ambulance unit arrives and transfer of patient care has been completed. If it is determined that medical service is not needed, ambulance response should be canceled according to "Cancellation of Ambulance Response," Policy #5400. In the event another 911 request for public safety services occurs while a first responder unit is caring for a patient, the scene commander shall use discretion and allocate existing resources to effectively manage the patient and cope with the secondary request for public safety services.
- C. First responders should be returned to service and be available to respond to other emergency calls once the responsibility for patient care is transferred to other EMS personnel. First responders may remain on the scene and assist as necessary to facilitate optimal patient care.

- D. Transporting EMS personnel must follow the Solano County EMS Policy #6110 addressing assumption of patient care. Other EMS personnel should avoid needless removal of dressings or splints applied by first responders unless:
 - 1. The seriousness of degree of injury must be reevaluated.
 - 2. The procedures or methods are considered inappropriate or could further injure the patient.
- E. Five or more casualties – For incidents involving five or more casualties, first responders shall initiate triage as necessary (refer to Solano County Multi-Casualty Incident (MCI) Response Plan, Policy #6180).

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Health & Social Services Department

Donald R. Rowe, Director

Public Health - Mental Health - Public Guardian
Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

Public Health Division
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EMERGENCY MEDICAL SERVICES

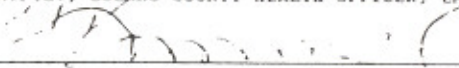
POLICY MEMORANDUM #5400

EFFECTIVE DATE: 11/01/92

APPROVED BY:


THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:


STEVEN J. KAMINSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

**SUBJECT: EMS FIRST RESPONDER CANCELLATION OF AMBULANCE
UNITS**

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.90, 1797.94,
1797.182, 1797.183, 1797.176, 1797.204, 1797.220

PURPOSE/POLICY: To define the appropriate procedure to cancel or change the response of an
ambulance unit.

I. CANCELLATION BY FIRST RESPONDERS

A first responder unit on the scene of an incident may cancel a responding ambulance unit upon
determination that there are no injuries or illnesses that require ambulance transport or the
patient(s) on scene refuse EMS services.

II. REDUCTION OF CODE BY FIRST RESPONDERS

A first responder unit on the scene of a medical incident may reduce a responding ambulance
from CODE 3 to CODE 2 upon determination that, in the best judgment of the first responder,
the illness or injury is not immediately life threatening or that the difference in CODE 3 and
CODE 2 response time would not impact patient outcome.

III. CANCELLATION OF FIRE FIRST RESPONDERS

Ambulance and/or law enforcement personnel arriving on the scene of a medical emergency prior to the fire first responders should cancel the fire first responder only if assistance is not needed and environmental hazards are nonexistent at the emergency scene.

- A. If EMT ambulance personnel are already on-scene and performing emergency care, first responder personnel shall offer assistance as needed.

IV. CANCELLATION OF A RESPONDING UNIT is to be based on sound medical judgment and common sense.



Health & Social Services Department

Donald R. Rowe, Director

Public Health - Mental Health - Public Guardian
Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

Public Health Division

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Health Officer

Deputy Director Health and Social Services

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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #5500

EFFECTIVE DATE: 06/01/93

APPROVED BY:

A handwritten signature in dark ink, appearing to read "Thomas L. Charron".

THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

A handwritten signature in dark ink, appearing to read "Allen Morini".

ALLEN MORINI, D.O., ASSISTANT EMS MEDICAL DIRECTOR

**SUBJECT: REQUIREMENTS FOR APPROVAL AS A GROUND AMBULANCE
SERVICE PROVIDER AGENCY**

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220

PURPOSE/POLICY:

To provide for the public health and safety to the citizens of Solano County by the establishment of effective standards and requirements for ambulance owners or ambulance service providers operating within Solano County.

- I. **AMBULANCE SERVICE PROVIDER PERMITS** – No ambulance owner or operator shall operate or cause to be operated, an ambulance service within Solano County unless a permit for such a service provider has been issued by the County Health Officer.
- II. **INITIAL APPLICATION PROCESS FOR AN AMBULANCE SERVICE PROVIDER PERMIT:**
 - A. New Ambulance Service Providers – Ground Ambulance services wishing to obtain a permit as an ambulance service provider who have not been servicing the County on or after March 1, 1993 shall be required to complete the following application process (Air Ambulances shall comply with Policy #5820) requirements as listed in Section III of this policy.
 1. **Need Justification** – The service shall provide to the EMS Office a need necessity justification regarding how the new service will benefit the existing Solano County EMS System.

2. Application Review – The application shall be submitted to the EMCC for public review, comment and recommendation to the Health Officer for action. Existing provider concerns – Existing ambulance service providers shall be given 45 days to respond, in writing, with any objections or concerns regarding ways the new service provider would negatively impact the existing EMS System.
3. EMCC – The EMCC shall advise the Health Officer regarding whether the new service provider application should be approved.
4. Costs – If the Health Officer agrees that the new ambulance service provider should be issued an ambulance service provider permit, the new service provider shall agree, in writing, to pay all of the following costs:
 - a. Costs to modify the County’s current system status plan to include a new service provider;
 - b. Any other appropriate additional costs to be incurred by the management of the CMED to allow for the additional ambulance service to be included in the Solano EMS System.
5. Board of Supervisor Approval – Prior to the EMS Office issuing any permit for a new ambulance service provider, the request for permit shall be submitted to the Board of Supervisors for their approval.
6. Exemptions from this Policy –
 - a. Ambulance services which are performing standby coverage at special events in accordance with Policy #5500.
 - b. Ambulance services responding into the County upon request from the Health Officer or his/her designee due to a disaster or other significant emergency in which there is a clear threat to the public health and safety or there is a clear need for medical mutual aid. Situations include in times of war or disaster declared by the President of the United States, the Governor of California, the Board of Supervisors of Solano County, or their successors, involving Solano County, the provisions of this policy may be temporarily waived by the County Health Officer or his/her designee.
 - c. Current ambulance service provider(s) currently working under a performance contract to provide emergency ambulance service consistent with community expectation within a dedicated ambulance zone within Solano County (excluding section II.B.7, 8, 9, 10 which shall be reported annually to the EMS Office.
 - d. Ambulance service providers who:
 - (1) Have successfully completed a “Request for Proposal” (RFP) process;
 - (2) Are selected by the Board of Supervisors to be responsible for an “exclusive operating zone” within the County;

- (3) Have signed a binding ambulance performance contract for provision of emergency ambulance service.
- e. Ambulance operations which involve picking up a patient or victim at a site outside Solano County for delivery to a destination within, through or outside the County.

III. INITIAL OR ANNUAL RENEWAL APPLICATION PROCESS

- A. **Application** – Applicants requesting renewal to operate as a prehospital service provider or ambulance service provider in Solano County shall submit in writing, on a form provided by the EMS Agency, an application with the following information:
 1. Name and description of the applicant.
 2. Business address and residence address of record of the applicant.
 3. Base location and/or prime posting position of each prehospital ALS/BLS response unit servicing the County.
 4. Trade or firm name or DBA as recorded.
 5. If a corporation, a joint venture, or a partnership or limited partnership, the names of all partners or the names of corporate officers, their permanent addresses and their percentage of participation in the business.
 6. A bond using any format approved by the County Auditor equal to 150% of the highest fine for performance failure and the annual costs to link with the CMED.
- B. **Applications shall be accompanied by:**
 1. **A statement of facts** showing the experience of the service provider in the operation of a prehospital ALS/BLS service and that the applicant is qualified to render efficient twenty-four (24) hour/day prehospital ALS/BLS service.
 2. **A statement signed by the applicant** that as a condition of the Health Officer's approval, applicant agrees to appear and defend all actions against the County arising out of the exercise of said approval, and shall indemnify and save the County, its officers, employees and agents harmless and from all claims, demands, actions or causes of actions of every kind and description resulting directly or indirectly, arising out of or in any way connected with the exercise of this approval. Documentation of Insurance naming Solano County as co-insured may meet this requirement.
 3. **Documentation of current insurance** naming Solano County as co-insured, to include:
 - a. Public liability insurance covering bodily injury, personal injury and property damage, to include commercial general liability and professional liability (malpractice) in an amount no less than five hundred thousand (\$500,000) for each occurrence or a verification of self-insurance for public safety agencies providing ambulance service.
 - b. Comprehensive automobile liability insurance covering bodily injury and property damage on owned, hired, leased and non-owned vehicles in an

amount no less than one million dollars (\$1,000,000) for each occurrence or a verification of self-insurance for public safety agencies providing ambulance service.

- c. Worker's Compensation Insurance as required by statute, **OR**

A signed indemnification statement, if not required to submit evidence of insurance naming the County as co-insured as part of the permit renewal process for ambulance operations in the unincorporated area of Solano County (Public Safety Agencies Only).

4. **A narrative description** of the proposed advanced life support operations (if applicable), including, but not limited to:
- a. Geographic area to be served to include a statement that the Ambulance Service Provider shall service the entire Zone(s) as prescribed by the Board of Supervisors.
 - b. Staffing patterns including a list of all prehospital care personnel currently certified to provide service.
 - c. In-house procedures for management and evaluation of ALS and BLS operations.
5. **A description of compliance** for each of the following program requirements:
- a. Ambulances must meet Title 13 requirements as established in California Code of Regulations, Chapter 5, Article 1.
 - b. All units must be equipped according to Solano county EMS Policy Memoranda 5600, 5700 and 5710 (EXCEPTION: non-transporting vehicles do not have to carry a gurney or related supplies and equipment, as determined by the Solano County Health Officer/Medical Director).
 - c. Applicant must document that it employs sufficient persons with required Solano County certification to provide twenty-four (24) hour coverage (state coverage patterns).
6. **Condition and Compliance Statement** – Application shall include a positive statement accepting each of the following conditions, and shall be accompanied by a statement describing compliance or plans for compliance with each criterion:
- a. **Maintenance of level of service:** Applicant does not foresee any reduction or change from level of service, vehicles, staff or capabilities in any area covered by this application. Any reduction or expansion of service shall be first approved, in writing, by the Health Officer or his designee before the permit shall be issued.
 - b. **Employee Orientation:** all employees will be adequately oriented to current Solano County Policies and Procedures, in a timely manner.
 - c. **ALS Staffing:** Each unit operating at an advanced life support level shall be staffed at the minimum by two persons, one of whom is certified as an Emergency Medical Technician – Paramedic by the County of Solano and

one of whom is certified as an Emergency Medical Technician – I. The alternative acceptable staffing also permitted is use of an additional EMT-P who is certified in Solano County in place of the required EMT-1.

- d. **BLS Staffing:** Each unit operating at a Basic Life Support level shall be staffed at the minimum by two persons, one of whom is certified as an Emergency Medical Technician (EMT-1) by the County of Solano.
- e. **Equipment and Supplies:** Applicant owns or has under its control and in good mechanical condition, required equipment to adequately conduct service in the area for which he is applying, which meets the requirements established by the California Vehicle Code, the Solano County Ambulance Ordinance, and regulations adopted pursuant thereto, the ALS program requirements and policies established by the Solano County Health Officer/Medical Director.
- f. **Record-Keeping:** Applicant and its employees will cooperate with the Solano County Emergency Medical Services Agency in the collection and analysis of patient care and other data necessary to an on-going evaluation of advanced life support operations. Necessary records and other information will be provided to the Solano County Health Officer/Medical Director (or designee) to allow assessment of ALS services. Appropriate confidentiality will be maintained.
- g. **Policies and Protocols:** Applicant agrees to follow all Solano County EMS Agency protocols, policies and procedures and comply with all applicable local, state and federal laws and regulations.
- h. **Decertification:** Applicant recognizes the right of the Solano County Health Officer/Medical Director to suspend or decertify personnel and agrees that persons so suspended or decertified will not be allowed to operate or function on an ambulance or other advanced life support unit.
- i. **Records:** Applicant agrees to complete such reporting mechanisms as required by the County.
- j. **Replacement of Medications:** Applicant agrees that it will not charge for drugs, medications, solutions or supplies used in any medical emergency when such materials are replaced by a hospital.
- k. **Right of Inspection:** Applicant accepts the right of the Solano County Health Officer/Medical Director (or designee) to inspect the records, facilities, vehicles, equipment, supplies and methods of operation whenever such inspections are deemed necessary. Each permitted prehospital ALS service, its equipment and premises, vehicle maintenance records, and records of calls shall be open to inspection by the Solano County Health Officer/Medical Director (or designee) during usual hours of operation.
- l. **A Statement Acknowledging the Acceptance of MediCare Assignment** for Emergency Calls received by Public Safety Answering Points (911 Centers).

- m. **A Statement agreeing to notify the EMS and EMCC**, in writing, 60 days before any increase of existing rates, and allow for public and EMS staff comment before implementing any increase in fees for EMS services rendered.
 - n. **Documentation of the relationship between the ambulance service provider and the CMED** regarding dispatch procedure and coordination of emergency operations.
 - o. **A signed “Acceptance of Conditions” statement** to the items listed above.
 - 7. A current itemized listing of all charges the Ambulance Service Provider will bill patients for services rendered.
 - 8. The Ambulance Service Provider’s Q. A. Plan for approval by the County Health Officer (or designee).
 - 9. A listing of each permitted ambulance unit that services Solano County.
 - 10. Signature sheet of EMT-Ps certified to administer and restock Class III medications.
 - C. Supporting documents should be attached and the proposal and application shall be signed by an individual with the authority to legally bind the applicant.
 - D. Public Record – All materials submitted in an application shall become public record and are subject to all laws and regulations applicable to public records.
 - E. Review and Approval – The Solano County Health Officer/Medical Director shall review new applications and require supporting documentation. Approval or disapproval shall be made in writing by the Solano County Health Officer/Medical Director to the applicant agency within thirty (30) days after receipt of all requirements and documentation.
 - F. Approval Condition – Approval of the Solano County Health Officer/Medical Director shall be conditional to the payment of permit fees, unless the agency is exempted from fee requirements per resolution of the Board of Supervisors.
 - G. Commencement of Services – Services shall commence within fourteen (14) days after approval, and issuance of a permit, if applicable, unless an extension is granted by the Solano County Health Officer/Medical Director.
 - H. Currently Licensed Agency Requirements – Each year, all agencies currently licensed and/or approved as prehospital ambulance service providers shall submit the information, statements, documentation, etc., identified in Section III of this policy by June 30th. Preference in the issuance of permits shall be given to established operators and permits shall be issued to new applicants when the Board of Supervisors finds that there is a public need and necessity for a new ambulance service proposed by the applicant, or as the result of a Request for Proposal (RFP) process at a contract renewal period specified by the Board.
- IV. **ISSUANCE** – If the County Health Officer determines that the standards and requirements are met by the applicant, he/she shall issue a permit. Permits shall be issued upon payment of a fee to the County Health Officer, the amount of such fee to be determined by the Board of Supervisors. Permits will be issued as either ALS Service Provider or BLS Service Provider.

All permits shall expire on the June 30th following their issuance, but may be renewed by the ambulance owner or operator upon payment of a renewal fee and re-submission of documents as listed in Section III of this Policy as determined by the Board of Supervisors.

- V. **REVOCATION** – Based upon evidence of significant or persistent violations of the standards and requirements set forth in State Regulations or County Policy, the County Health Officer may suspend or revoke a permit issued hereunder. Any suspension shall not exceed four (4) weeks duration. Any revocation shall be permanent, provided, however, that an ambulance operator whose permit has been revoked may reapply for an ambulance service permit as a new operator after three (3) months from the date of revocation. No action to suspend or revoke a permit shall be taken unless the ambulance operator has been given timely notice of the charges and a fair hearing before the County Health Officer. Any ambulance operator whose permit is suspended or revoked by the County Health Officer may appeal such decision to the Board of Supervisors within ten (10) Days.
- VI. **ADDITIONAL OPERATIONAL PROCEDURES** – The County Health Officer may prescribe additional operating procedures and requirements including but not limited to establishment of zones, response time standards, and additional EMS System Policies in order to ensure effective, safe and rapid pick up and delivery of patients, and to provide effective coordination of operations between ambulance operators, County and local authorities and medical personnel at hospitals.

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EMORY COWAN, LPCC, LMHC
Director

EMERGENCY SERVICES BUREAU
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DEPARTMENT OF HEALTH & SOCIAL SERVICES

Public Health Division



SOLANO
COUNTY

PRANAV SHETTY, MD, MPH
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POLICY MEMORANDUM 5510

Implementation Date: July 1, 2013

Revised Date: November 1, 2025

REVIEWED/APPROVED BY:

A blue ink signature of Pranav Shetty, MD, MPH.

PRANAV SHETTY, MD, MPH, EMS AGENCY MEDICAL DIRECTOR

A blue ink signature of Benjamin Gammon, EMT-P.

BENJAMIN GAMMON, EMT-P, EMS AGENCY ADMINISTRATOR

**SUBJECT: NON-EXCLUSIVE AMBULANCE PERMIT AND INSEPCTION
PROCESS (BLS AND CCT)**

AUTHORITY: Health and Safety Code, Division 2.5, Sections 1797.204, 1797.220,
1708(b), 1798.170

Solano Emergency Medical Services Cooperative (SEMSC)
Resolution 23-001 and 25-001

PURPOSE:

To establish requirements and a process to permit non-exclusive Basic Life Support (BLS) and Critical Care Transport (CCT) providers to allow Interfacility Transports (IFT) within Solano County.

I. BLS AND CCT PERMIT REQUIREMENT

A. Per SEMSC Resolution 23-001 and 25-001, all non-exclusive BLS and CCT providers that wish to provide services that originate within Solano County must be permitted by Solano County EMS Agency.

1. Advanced Life Support (ALS) transport services by another provider than the current Exclusive Operations Area (EOA) provider is not permitted.

- B. This permit allows for BLS or CCT Interfacility Transfer (IFT) and BLS standby operations within Solano County ONLY.
 - 1. Any request for transport from a scene of an incident to an Emergency Department is not allowed with this permit. This type of call will be handed over to the current Solano County ALS EOA Provider.

II. NON-EXCULSIVE BLS PROVIDER INITIAL PERMIT APPLICATION AND INSPECTIONS

- A. The requirements for initial non-exclusive BLS provider are as follows:
 - 1. Completed application (Form will be provided). The application documents include the following:
 - a. Company name
 - b. Company's designated point of contact and information
 - c. Type of company, e.g. sole proprietorship, partnership, corporation, etc.
 - d. Portfolio of company's experience in the field of EMS and transportation of patients
 - e. Description of each ambulance, including digital pictures and Vehicle Identification Numbers (VIN) of those vehicles to be used in Solano County
 - f. Copies of current California Highway Patrol Inspection Reports for each unit to be used in Solano County
 - g. Current copies of appropriate license(s) and/or permits required by State law and regulations
 - h. A current roster of staff members, with certification/license numbers and expiration dates
 - i. Company orientation and training materials used to orient new staff
 - j. Copies of most recent financial statement, e.g. revenue, expenditures, balance sheet, etc.
 - k. Evidence of current insurance
 - l. Evidence of current Worker's Compensation insurance
 - 2. Each unit intended for use in Solano County will undergo a unit equipment inspection.
 - a. The unit equipment inspection is performed initially by the provider applying for the permit using the unit inspection checklist (Attachment B).

- i. The checklist will be signed by the provider attesting the unit has been inspected and meets the equipment requirements.
 3. Pay the appropriate non-exclusive BLS provider application fee as set forth in Solano EMS Policy 3000, EMS Fees.
 4. Pay the appropriate BLS unit inspection fee for each unit to be utilized in Solano County as set forth in in Solano EMS Policy 3000, EMS Fees.
- B. The EMS Agency will have 30 days to review and issue an approval or denial for the BLS permit application.
 1. If the application is denied, a list of deficiencies will be stated to the provider.
 2. The provider will then have 30 days to remedy the deficiencies and resubmit.
 - a. If the provider cannot remedy the deficiencies after the second attempt, the provider's application will be fully denied and cannot reapply for a period of one year.
- C. After the non-exclusive BLS permit is approved, the permit will be valid until the end of the calendar year issued.
 1. If the permit is issued within 90 days of the end of the calendar year, the permit will be valid until the end of the next calendar year.
- D. All permitted providers will adhere to all State statutes and regulations and Solano County EMS resolutions, policies, and protocols.

III. NON-EXCULSIVE BLS PROVIDER PERMIT APPLICATION AND INSPECTION RENEWAL

- A. Non-exclusive ambulance permits are only valid for one calendar year. The process for permit renewal is as follows:
 1. Submit will documentation as stated in Section II(A) of this policy.
 2. Pay all appropriate permit renewal and unit inspection fees as set forth in Solano EMS Policy 3000, EMS Fees.
 3. All documentation and fees are due prior to the end of the current calendar year.
 - a. If all documentation and fees are not received prior to the end of the current calendar year, the provider will not be allowed to provide any services in Solano County until all documentation and fees are received.

IV. NON-EXCLUSIVE CCT PROVIDER INITIAL PERMIT APPLICATION AND INSPECTION

- A. A non-exclusive CCT permit is considered a separate supplemental permit to any existing permit such as non-exclusive BLS or the current Solano County EOA Provider.
- B. The process to apply to obtain a non-exclusive CCT operating permit is as follows:
 - 1. Obtain a valid current non-exclusive BLS operating permit in Solano County OR are the current Solano County EOA Provider.
 - 2. Submit the Supplemental Application to Provide CCT Services.
 - a. Required documentation is listed on the application.
 - 3. Each unit intended for use in Solano County will undergo a unit equipment inspection as stated in Section II(A)(2) of this policy.
 - 4. Pay the appropriate non-exclusive CCT provider application fee as set forth in Solano EMS Policy 3000, EMS Fees.
 - 5. Pay the appropriate CCT unit inspection fee for each unit to be utilized in Solano County as set forth in in Solano EMS Policy 3000, EMS Fees.
- C. The EMS Agency will have 30 days to review and issue an approval or denial for the CCT permit application.
 - 1. If the application is denied, a list of deficiencies will be stated to the provider.
 - 2. The provider will then have 30 days to remedy the deficiencies and resubmit.
 - a. If the provider cannot remedy the deficiencies after the second attempt, the provider's application will be fully denied and cannot reapply for a period of one year.
- D. After the non-exclusive CCT permit is approved, the permit will be valid until the end of the April on odd numbered years.
 - 1. If the permit is issued within 90 days of the end of April on the odd numbered year, the permit will be valid until the end of the next end of April on the odd numbered year.
- E. All permitted providers will adhere to all State statutes and regulations and Solano County EMS resolutions, policies, and protocols.

V. NON-EXCLUSIVE CCT PROVIDER PERMIT APPLICATION RENEWAL AND RENEWAL

- A. CCT Permit renewal will take place every two years at the end of April on odd numbered years.
- B. The renewal process will be the same as stated in Section IV(B) of this policy.
- C. Pay all appropriate permit renewal and unit inspection fees as set forth in Solano EMS Policy 3000, EMS Fees.
 - 1. CCT unit inspection fees are due annually with the payment of the BLS non-exclusive permit fee and BLS unit inspection fees.
- D. All documentation and fees are due prior to the end of the current permit term.
 - 1. If all documentation and fees are not received prior to the end of the current term, the provider will not be allowed to provide any services in Solano County until all documentation and fees are received.

VI. PERSONNEL, VEHICLE, AND DATA REPORTING STANDARDS

- A. Providers will ensure that all ambulance personnel are currently licensed or certified by the appropriate licensing or certifying authority. Any EMT employed and used to transport patients in Solano County must be registered with Solano County EMS.
- B. Providers will ensure that any vehicle used in Solano County complies with all applicable federal, state, and local laws and regulations and SEMSC resolutions, protocols, and policies.
- C. Providers shall have an electronic Patient Care Report (ePCR) system that is compliant with the most recent version of NEMSIS. The ePCR system shall be linked with Solano County EMS's data depository system to be in compliance with state EMSA reporting requirements.
- D. Providers will ensure that any vehicle used in Solano County has the capability for two-way radio and cellular communication with the operators' respective dispatch centers and Solano County hospitals.

VII. ADDITIONAL COMPLAINT AND INVESTIGATIONS

- A. BLS and CCT Providers will strictly adhere to all state statutes and regulations, SEMSC Resolution 23-001, and Solano County EMS Policies regarding permitting and IFTs (Reference Policy 7200).
- B. CCT Providers will strictly adhere to SEMSC Resolution 25-001 as well as the items listed in Section VII(A).
 - 1. All calls that do not meet the criteria for a ALS LOSOP or CCT patient in Solano County will be transported by the current ALS EOA provider.

- C. If a provider is found in violation of any item listed above, the provider may be penalized in accordance with SEMSC Resolution 23-001 Section 9.
- D. BLS and CCT Providers shall comply with any requests for, but not limited to, ePCRs, Computer Aided Dispatch (CAD) logs, certification/licensure records, billing records, and dispatch records during the course of an investigation.
- E. The EMS Agency maintains the right to perform unannounced inspections of Provider's facilities, and any ambulance vehicles authorized for use in Solano County for the purpose of determining compliance with applicable federal, state, and local laws and regulations, and SEMSC resolutions, protocols, and policies. Inspections may occur when a unit is waiting for a call or after a call. Inspections will not interfere with patient care.
 - 1. CCT units may be required to provide additional evidence they are in compliance with all SEMSC Resolutions and policies as it relates to CCT patient criteria versus ALS and ALS RN type calls.

VIII. DISASTER SUPPORT

- A. In the event of a locally declared disaster, Providers will fully cooperate and provide response to the disaster when requested by the Solano County Medical Health Operational Area Coordinator (MHOAC) and/or the CalOES Region II Regional Disaster Medical and Health Specialist (RDMHS).
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Solano County Health and Social Services Department

Mental Health Services
Public Health Services

Adult and Child Services
Substance Abuse Services

Eligibility and Employment Services
Public Guardian/Conservator

Donald R. Rowe, Director

Public Health Division
Thomas L. Charron, M.D., M.P.H.,
Health Officer/EMS Medical Director
Deputy Director, Health & Social Services

EMERGENCY MEDICAL SERVICES
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POLICY MEMORANDUM #5520

EFFECTIVE DATE: 04/27/94

APPROVED BY:

Thomas L. Charron MD
THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

Allen J. Morini, D.O.
ALLEN J. MORINI, D.O., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: COMMUNITY PERFORMANCE STANDARDS FOR AMBULANCE PROVIDERS WHEN RESPONDING TO EMERGENCY REQUESTS FOR MEDICAL SERVICES

AUTHORITY: SOLANO COUNTY AMBULANCE ORDINANCE #1457; CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.204, 1797.220

PURPOSE/POLICY:

All ambulances responding to medical emergencies in Solano County must comply with the following community performance standards (excluding any period of time classified as a period of "unusual system overload.").

I. DEFINITION

- A. Emergency Call (Service): means the functions involved in responding to a request for an ambulance to transport or assist person(s) in apparent sudden need of medical attention. An emergency call is unplanned and non-scheduled and requires the dispatch of an ambulance immediately after the receipt of a request. The ambulance may respond in a "CODE 2" or "CODE 3" mode. Emergency calls are not limited to only the prehospital environment. Emergency calls shall be classified using the Priority Classification System as listed in Section II.
- B. Scheduled Call (Service): means the functions involved in responding to a planned request for an ambulance to transport person(s) needing medical transportation. The ambulance shall respond in a "CODE 1" mode. All requests for medical transport are considered an emergency call unless the party requesting ambulance services has made

the previous arrangement of scheduling the ambulance at least one (1) hour before the ambulance is actually dispatched..

- C. “CODE 3” Operation (CODE 3): is a term used to describe the use of red warning lamps and siren as permitted by the California Vehicle Code Section 21055 if the vehicle is being driven in response to an emergency call. “CODE 3” is justified only when initially responding to an emergency call or returning to a hospital with a patient(s) when speed in obtaining emergency medical care appears essential to save a life, prevent undue suffering or to prevent disability. “CODE 3” transportation of a stabilized patient is seldom necessary and often undesirable. Higher priority Code 3 emergency calls may be re-routed by the CMED if a higher medical priority is determined by the EMD(s) dispatching ambulances.
 - D. “CODE 2” Operation s(CODE 2): is a term used to describe an immediate response of an ambulance without lamps and siren as permitted by Section 21055 of the California Vehicle Code. A “CODE 2” response is immediate, non-scheduled, and does not require use of red warning lamps and siren, but the ambulance is immediately responding to a request for medical services. Code 2 calls may be re-routed to another higher priority Code 3 emergency call if a higher medical priority is determined by the EMD(s) dispatching ambulances.
 - E. “CODE 1” Operation (CODE 1): is a term used to describe the mode of ambulance response to a scheduled call. The ambulance responding is not responding immediately and the patient being transported is stable, not needing aggressive EMS intervention. Scheduled transfers shall be logged a “CODE 1” in medical records even if the ambulance service dispatcher requests the ambulance personnel to immediately respond at the designated or prescribed arranged transport time. EMTs, upon arrival of a scheduled call, who assess the patient(s) clinical status as needing aggressive EMS intervention, shall log their mode of response to a medical facility as “CODE2” or “CODE3” as appropriate. Code 1 calls may be re-routed to another higher priority Code 3/2 emergency call if a higher medical priority is determined by the EMD(s) dispatching ambulances.
- 1. Urban Area: is a term used to describe the population density of a grid area used by the Solano County EMS Office to monitor ambulance response time performance. Urban areas are determined by successfully meeting one or all of the following characteristics:
 - a. The area is within the incorporated area of a city within Solano County.
 - b. The projected population density of the area within the grid is more than 100 people within the grid.
 - c. The area is declared as an Urban Area by the Health Officer or his/her designee after public review and input from the County EMCC.
 - 2. Rural Area: is a term used to describe the population density of a grid area used by the Solano County EMS Office to monitor ambulance response time performance.

Rural areas are determined by successfully meeting one or all of the following characteristics:

- a. The area may or may not be within the incorporated area of a city within Solano County.
 - b. The projected population density of the area within the grid is 25 – 99 people within the grid.
 - c. The area is declared a Rural Area by the Health Officer or his/her designee after public review and input from the EMCC.
- H. Remote Area: is a term used to describe the population density of a grid area used by the Solano County EMS Office to monitor ambulance response time performance. Remote areas are determined by successfully meeting one or all of the following characteristics:
1. The area is in the unincorporated area of Solano County (or other border regions of counties, Solano Ambulances routinely responding outside the County).
 2. The projected population density of the area within the grid is less than 25 people within the grid.
 3. The area is declared a Remote Area by the Health Officer or his/her designee after public review and input from the EMCC.
- I. Ambulance Response Time: The mathematical time interval (measured in minutes) from when a request for an emergency call is made known to the dispatcher of an ambulance service (or Central Medical Emergency Dispatch [CMED]) until the arrival of the ambulance at the scene of a medical emergency and the EMT's exit from the ambulance.
- J. EMS System Response Time: The mathematical time interval (measured in minutes) from when a primary Public Safety Answering Point (PSAP) initially receives a request for EMS Services until the arrival of the ambulances at the scene of a medical emergency and the EMTs exit from the ambulance.
- K. Ambulance Dispatch Interval: The mathematical time interval (measured in seconds) from when a request for an emergency call is received by the dispatcher of the ambulance service (or CMED) until the ambulance unit is enroute and responding to the call.
- L. Unusual System Overload: A situation which could not have been predicted based upon past call volume data where over 66.6% (2/3) of the ambulances scheduled for that hour in a Zone within the current system status plan are assigned to an emergency call. An Unusual System Overload may include any of the following:
1. A declared disaster or Multi-Casualty Incident (MCI) in Solano County or in a neighboring jurisdiction which has requested assistance from the County.
 2. A major fire in an ambulance zone where the prime ambulance service provider is a Fire Department.

II. EMERGENCY CALL PRIORITY CLASSIFICATION SYSTEM:

Priority Level	Application	Code Out	Min Unit Capab.	Disp of 1 st Responders	Base Rate Charged To patient
1	An EMS call for a presumed life threatening emergency. This type of call shall be determined by an EMD using approved medical protocols where the EMD determines that there is a high priority that the caller(s) needing EMS services has (or will have) airway, respiratory or cardiac dysfunction and EMT-P intervention is needed. Pre-arrival instructions are clinically indicated and may positively impact the individual(s) requesting services. (Example: An emergency call in which an EMD can confirm acute respiratory distress and/or unconsciousness).	3	ALS	Yes	EMT-P (ALS)
2	An EMS call for a presumed non-life threatening emergency. This type of call shall be determined by an EMD using approved medical protocols where the EMD determines that a patient's status is unstable and may result into cardio-respiratory failure and/or the EMD is unable to ascertain the clinical status of the patient. EMT-P intervention is required, and pre-arrival instructions may/may not be indicated in this type of call. (Example: A 9-1-1- call in which an EMD determines that the patient is not in severe respiratory distress but is exhibiting signs/symptoms of a heart attack (AMI).	3	ALS	Yes/No dependent upon Medical Protocols the EMD uses.	EMT-P (ALS)
3	An EMS call for a presumed non-life threatening, but emergent condition. This priority requires an immediate dispatch for reasons other than a threat to life or limb (including lack of information from the caller). This type of call includes any nonscheduled request where the patient's intended destination is an acute care facility. The response shall be made by the closest available ALS ambulance. Specifically these calls cannot be "stacked" or "held" and are classified by an EMD as an emergency case possibly needing EMT-P services. (Example: A 9-1-1-call in which the patient is conscious and in no respiratory distress but exhibits need for an immediate EMS response; e.g., fractured hip, pulseless extremity, painful injury, etc).	3	ALS	No	EMT-P (ALS)
4.	An EMS call, non-scheduled response for a presumed life threatening emergency at a medical facility where a patient is under the direct care of a MD wishing immediate transport to another medical facility that has a higher level of care. This type of call shall be determined by an EMD using approved medical protocols in which the EMD determines that time for transport is of the essences, and where the sending MD may or may not have arranged appropriate medical staffing for the transport unit. Pre-arrival instructions are not clinically indicated but the EMD may provide advice regarding the proper ambulance to send. (Example: Obstetric emergency being transferred to a perinatal center).	3	ALS	No; (MD may or may not elect to send other medical staff with the patient being transported.	EMT-P (ALS)
5	An EMS call, non-emergency and unscheduled (request for transport services occurred with one hour of the unit being dispatched) for a presumed non-life threatening call at a medical facility where a patient is under the direct care of a MD arranging transport to another medical facility. This type of call shall be determined by an EMD using approved medical protocols in which the EMD determines the sending MD is appropriately requesting a BLS unit, the patient is determined to be stable, and that response time of the responding unit shall not negatively impact the patient. The closest ambulance need not be dispatched for this type of call, and that the requestor may utilize a specific type of ambulance service because the ambulance response has been predetermined by a pre-existing agreement or relationship with the service. The EMD handling this type of call may dispatch a unit in the priority 1, 2 or 3 mode if the requestor cannot comply with Policy 5540 or the EMD determines the patient is unstable. (Example: Nursing home staff requesting transport for a stable patient needing to be seen in an emergency department).	2	BLS or ALS	No	EMT-I (BLS)
6	Code 2 non-emergency response for a scheduled ambulance transport which is to be picked up outside of the County of Solano (Example: self-explanatory)	2	BLS or ALS	No	EMT-I (BLS)
7	Special event ambulance standby (Example: self-explanatory).	NA	ALS or BLS	No	N/A
8	Special ambulance transport in which specialized medical staffing will assume all patient responsibilities and medical equipment has been prearranged (e.g., NICU				

	transports, Spinal Team Transports, Cardiac Cath Surgery Transports). (Example: self-explanatory).	2 or 3	BLS or ALS	No	EMT-1 (BLS)
9	An immediate request for ambulance transport services where an EMD is able to determine by using call prioritization medical algorithm that EMT-P services are not needed and that a Basic Life Support ambulance can effectively and appropriately manage the patient's medical needs. This type of call may be received from a 9-1-1-Primary PSAP, Public Safety Officer on scene, medical facility, or a member of the general public requesting immediate ambulance transport services. The EMD shall advise the requesting party of the determination that an EMT-P ambulance is not necessary and a less expensive BLS unit shall respond (Example: Kaiser triage nurse requesting immediate transfer of a patient to an emergency department).	2	BLS	No	EMT-1 (BLS)

III. AMBULANCE RESPONSE TIME STANDARDS

- A. Only emergency calls shall be reviewed by the EMS Agency to assure compliance with community performance standards.
- B. Thomas Brothers Map coordinates shall be used as grids for determination as to whether ambulance response time standards are consistent with community performance standards (see Attachment 2)
- C. The following is the Ambulance Response Time Standard for "CODE 3" emergency calls in the Solano County EMS System:
 1. Nine (9) minutes for areas designated as Urban Areas. Until Performance Contracts of the current Ambulance Service Providers are signed with the Board of Supervisors, Ambulance Services shall insure that no less than 90% of the Code 3 calls in an urban area are consistent with the nine (9) minute Response Time Standard. By July 1, 1994, 93% of the emergency calls shall comply with the nine (9) minute Response Time Standard. By December 1, 1994, 94% of the EMS calls shall comply with the nine (9) minute Response Time Standard. By December 31, 1994, 95% of the emergency calls shall comply with the nine (9) minute Response Time Standard.
 2. Fifteen (15) minutes for areas designated as Rural Areas. Until Performance Contracts of the current Ambulance Services Providers are signed with the Board of Supervisors, Ambulance Services shall insure that no less than 90% of the Code 3 calls in a rural area consistent with the fifteen (15) minute Response Time Standard (excluding emergency calls in which the EMS Office has determined that there is good cause for the performance failure). By December 31, 1993, 92% of all Code 3 calls shall comply with the fifteen (15) minute response time standard. By July 1, 1994, 93% of the emergency calls shall comply with the fifteen (15) minute response time standard. By December 1, 1994, 94% of the EMS calls shall comply with the fifteen (15) minute response time standard. By December 31, 1994, 95% of the emergency calls shall comply with the fifteen (15) minute response time standard.
 3. Twenty (20) minutes for areas designated as Remote Areas.

IV. AMBULANCE STAFFING AND CAPABILITY STANDARDS

ALS ambulances, exclusively, shall respond to all CODE 2 and/or CODE 3 emergency calls which are classified as Priority 1, 2, 3 or 4.

Exemptions to this standard are restricted to the following cases:

- A. BLS ambulances may be dispatched to an emergency call when the EMAD/CMED is using a “call prioritization” system approved by the Health Officer or his/her designee which allows BLS units to respond to appropriately classified emergencies.
- B. Dispatch of BLS units to Priority 1 – 4 emergency calls may occur during periods of “Unusual System Overload” or if a medical disaster has been declared.
- C. Emergency Hospital Inter-facility Transfers where hospital staff accompanies the patient and assumes total patient care during the transfer.
- D. Transfer calls in which an EMD, using approved call priorities and standards, determines an EMT-P ambulance is not an appropriate use of medical resources (e.g., Priority 9 requests for immediate ambulance transport services).

V. AMBULANCE DISPATCH INTERVAL STANDARDS

The following is the Ambulance Dispatch Interval Standard for emergency calls in Solano County:

120 Seconds for all “CODE 2” and “CODE 3” emergency calls.

VI. NOTIFICATION OF PERFORMANCE FAILURES

- A. Daily Review: Ambulance services management should daily review its service compliance with the Standards of Sections II and III listed above.
- B. Daily Review & Submission of Unusual Occurrence Form: The ambulance service review of each performance failure should be submitted to the EMS Office within 72 hours of each incident. The following performance failures shall be submitted to the EMS Office no later than three (3) working days from the date of the incident:
 - 1. Any CODE 3 calls in which response time exceeded the standard for population density.
 - 2. Any CODE 3 response where the unit originated outside its normal zone of responsibility.
 - 3. Any BLS unit dispatched to a CODE 3 call.
 - 4. Any CODE 1 call in which the ambulance responded back to a receiving facility CODE 3 or CODE 2, or any code we call in which the transport team elected to change the destination because of the patient’s deteriorating status.
 - 5. Any emergency call where an accident or mechanical failure of the ambulance occurred while enroute.
 - 6. Any call in which a mechanical failure occurred while transporting a patient.
 - 7. Any emergency call in which the unit responding took more than 120 seconds to go enroute.

SUBJECT: Community Performance Standards for Ambulance
Service Providers when Responding to Emergency
Requests for Medical Service

Policy: 5520
Date: 04/27/94

8. Any call where a citizen complaint or normal Q. A. follow-up, submitted to the EMS Office, results in a reported Unusual Occurrence.
- C. EMS Evaluation: EMS Staff shall evaluate each Unusual Occurrence and determine whether there is good cause for the performance failure based upon the analysis submitted by the ambulance service. Calls that are evaluated as significant performance failures without good cause shall be assigned a fiscal penalty. Performance failures shall be reviewed by the Compliance Sub-Committee for final resolution and disposition.

VII. DETERMINATION OF GRID RESPONSE TIME CHARACTERISTICS

- A. Basic Grid Unit – Thomas Brother’s Map coordinate grids shall be the basic elemental unit for an ambulance response zone.
- B. Grid Classification – Each Thomas Brother’s Map Grid shall be classified as an urban, rural or remote area (see Attachment 2).
- C. Grid Classification Change – Grid Classification may be changed dependent upon call volume, rates, and changes in population demographics associated with the grid. EMS Staff, ambulance service providers and/or general public may address in the public forum of the EMCC a reclassification of a grid. It is the responsibility of the Health Officer or his/her designee to make the final determination of an EMS System Grid if consensus cannot be achieved by the EMCC.

SUBJECT: Community Performance Standards for Ambulance
Service Providers when Responding to Emergency
Requests for Medical Service

Policy: 5520
Date: 04/27/94

SOLANO COUNTY EMS OFFICE – UNUSUAL OCCURRENCE REPORT
Ambulance Dispatch Failure Initial Investigation Summary

Ambulance Service: _____

Date/Time of Incident: _____ PCR # _____

Date of the Report/Investigation: _____ PSAP Dispatch # _____

Location of Call/Occurrence: _____ Map Coordinates: _____

Incident Category: (Check ALL that apply)

- | | |
|--|--|
| ☐ Code 3 call with > 9 min response in an urban area | ☐ Accident/Mechanical Failure Enroute to an Emergency Call |
| ☐ Code 3 call with > 15 min response in a rural area | ☐ Accident/Mechanical Failure while transporting a patient |
| ☐ Code 3 call with > 20 min response in a remote area | ☐ Emergency call with unit responding taking > 120 sec to go enroute. |
| ☐ Code 3 response out of normal operating zone | ☐ Other _____ |
| ☐ BLS unit dispatched on a Code 3 call | |

Snapshot status of all other ALS units in the zone at the time of incident:

Assessment of Probably Cause Code # _____ (see EMS Code List)

Effect on Patient/Affected Party (Note: how and by whom confirmed; enter not applicable or unknown as appropriate).

Corrective Actions Taken (note when, by whom and how/if documented; enter not applicable or unknown as appropriate).

Further Actions Planned (note when, by whom and how; enter not applicable or unknown as appropriate).

Report initiated by (Name) _____ Title: _____

Investigator Recommendations: ☐ Closure ☐ Further Follow-up by EMS Date of Submission: _____

TO BE COMPLETED BY THE SOLANO EMS OFFICE

Date Received at EMS Office: _____ Log Number: _____

QA Incident Category: ☐ Category 1 ☐ Category 2 ☐ Category 3

EMS Agency Follow-up

☐ Accept investigator findings and recommendations (incident closed) Signature: _____

☐ Initiate further EMS Agency Action

☐ Refer to EMCC Contract Compliance Sub-Committee Date: _____

CONFIDENTIAL (NOT part of the patient medical record)



Health & Social Services Department

Donald R. Rowe, Director

Public Health Division - Mental Health - Public Guardian
Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

Public Health Division
Thomas L. Charron, M.D., M.P.H.
Health Officer
Deputy Director Health and Social Services

EMERGENCY MEDICAL SERVICES
717-B Missouri Street
Fairfield CA 94533
(707) 421-6685
FAX (707) 421-6682

EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #5540

EFFECTIVE DATE: 5/26/93

APPROVED BY:

Thomas L. Charron
THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

Allen J. Morini, D.O.
ALLEN J. MORINI, D.O., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: IMMEDIATE REQUEST TO DISPATCH A BLS AMBULANCE FOR PATIENT TRANSFER

AUTHORITY: SOLANO COUNTY AMBULANCE ORDINANCE # 1457.; CALIFORNIA HEALTH & SAFETY CODE 1797.204, 1797.220

PURPOSE/POLICY:

To provide for the public health and safety of the citizens of Solano County by establishing standards for the dispatch of Basic Life Support (BLS) ambulance for patient transport under the care and direction of certified EMTs and to ensure that BLS ambulances are not inappropriately dispatched to life threatening emergencies.

BACKGROUND INFORMATION:

Based upon their familiarity with the patient's condition and the need to transport that patient to a higher level of care (acute care hospital), there are situations where private physicians may desire transport of the patient in a BLS ambulance; thus negating the initiation of an ALS response in the pre-hospital setting.

PROCEDURE:

The private physician (or his/her agent) may request a (BLS) ambulance to perform an unscheduled transport by adhering to the following procedure, thus establishing a mechanism for the private physician to have access to ambulances dedicated to the EMS system; but allows for clinical discretion in limiting costly ALS response and unnecessary medical services.

- I. TO REQUEST A BLS AMBULANCE FOR IMMEDIATE TRANSPORT via CMED (or a private ambulance), the Health Care Professional must:

- A. Establish that the patient is clinically stable within his/her pre-existing medical condition for immediate BLS ambulance transport. The health care professional requesting the immediate BLS transport shall provide the patient's vital signs (including, HR, B/P, & RR) and assurance that the patient will not need immediate pre-hospital ALS intervention until the patient arrives at his/her intended destination (the patient's MD wishes no ALS intervention and will verify that the patient's clinical status will not change until the next available BLS ambulance is dispatched). It is recognized that many patients under the care of a health care professional need transport via BLS ambulance and have long-term chronic illness or terminal disease associated with their need for immediate BLS transport. It is the responsibility of the medical professional requesting immediate BLS ambulance transport to determine that the patient condition does not require immediate pre-hospital ALS intervention.
 - B. Verbally assure the Dispatcher that the receiving facility has agreed to accept the patient, and all necessary information and paperwork are ready to accompany the patient. The Health Care Professional shall inform the Dispatcher that the transfer by BLS ambulance creates no medical hazard to the patient. The Dispatcher shall note the name of the Health Care Professional ordering an immediate BLS ambulance in his/her dispatch log.
- II. *IF ANY PART OF THIS INFORMATION IS UNAVAILABLE OR MISSING, THE EMERGENCY MEDICAL DISPATCHER (EMD) HAS THE RIGHT AND RESPONSIBILITY TO:*
- A. Dispatch an EMT-P ambulance to provide emergency care and transportation of the patient OR
 - B. Establish a three (3) way call with the Base Hospital Physician on duty at the Base Hospital designated for the area where the ambulance must respond to notify him/her of the case and the medical professional requesting a BLS ambulance. The final decision to respond an ALS or BLS ambulance rests with the Base Physician.
- III. *EMS calls where Physicians (or their agents) elect NOT to abide by this Policy , and initiate orders that prohibit the dispatch of an ALS ambulance, shall be subject to retrospective review by the EMS office to ascertain that a patient's clinical status did/did not deteriorate because the physician refused to comply with Federal and State "patient dumping laws." Any alleged violations of Local EMS Agency Transfer Protocols or Guidelines shall be evaluated by the Local EMS Agency. If the Local EMS Agency concludes that a violation has occurred, it shall take whatever corrective action it deems appropriate within it's jurisdiction, including referrals to either State Department of Health Services or the District Attorney under Section 1798.206 and 1798.208.*
-



Health & Social Services Department

Donald R. Rowe, Director

Public Health - Mental Health - Public Guardian
Adult and Child Services - Eligibility & Employment Services
Substance Abuse Services

Public Health Division
Thomas L. Charron, M.D., M.P.H.
Health Officer
Deputy Director Health and Social Services

EMERGENCY MEDICAL SERVICES
717-B MISSOURI AVENUE
Fairfield, CA 94533
(707) 421-6645

EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #5550

EFFECTIVE DATE: 11/1/92

APPROVED BY:

Handwritten signature of Thomas L. Charron in black ink.

THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

Handwritten signature of Steven J. Kaninski in black ink.

STEVEN J. KANINSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: EMS COVERAGE FOR SPECIAL EVENTS

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220

PURPOSE/POLICY:

To provide the procedures and requirements for authorization of EMS coverage for a special event.

I. DEFINITIONS:

Special Event: A pre-scheduled happening that, due to either the number of persons expected to be in attendance or the potential for injury during the activity, may require medical standby.

Permitted Provider: an ambulance service provider, permitted by Solano County EMS, that is based inside the county and responsible for ambulance coverage of a special event.

Non-permitted Provider: an ambulance service provider, non-permitted by Solano county EMS, that is based outside the county and is responsible for ambulance and/or first responder coverage of a special event.

Advanced Life Support (ALS): "Advanced Life Support" means special services designed to provide definitive prehospital emergency medical care, including, but not limited to, cardiopulmonary resuscitation, cardiac monitoring, advanced airway management, intravenous therapy, administration of specified drugs and other medicinal preparations, and other specified techniques and procedures administered by authorized personnel under the direct supervision of a Base Hospital as part of a local EMS system at the scene of an emergency, during transport to an acute care hospital, during Interfacility transfer, and while in the emergency department of an acute care hospital until responsibility is assumed by the emergency or other medical staff of that hospital.

Basic Life Support (BLS): “Basic Life Support” means emergency first aid and cardiopulmonary resuscitation procedures which, as a minimum, include recognizing respiratory and cardiac arrest and starting the proper application of cardiopulmonary resuscitation to maintain life without invasive techniques until the victim may be transported or until Advanced Life Support is available.

II. TYPES OF EMS COVERAGE

- A. ALS Coverage – All special events requiring standby ambulance service shall be capable of providing ALS services or provide an action plan approved by the EMS Office to have immediate ALS ground or air response if a BLS ambulance is posted at the event.
- B. BLS Coverage – A BLS ambulance shall be allowed for all special events that do not require an ambulance due to the requirements of the special event sponsor’s insurance carrier or the bylaws of the special event sponsor. The BLS unit shall be considered a first responder, and shall have a pre-arrival plan for ALS response as described in II.A above.
- C. Transport of Patients from the Special Event – The transport of any patient(s) from the scene of a special event shall be made by the ambulance provider assigned to the area encompassing the special event except as allowed by this policy.
- D. Non-permitted Ambulance Providers
 - 1. Non-permitted ALS providers wishing to provide special event coverage shall be required to obtain temporary accreditation of Advanced Life support(ALS) personnel who are not currently accredited within the County who shall document assessment and care on the standard medical records as required by the County.
 - 2. If an ALS ambulance is to be provided at the special event, the temporary permit of the ALS Ambulance service provider not currently approved within the County shall be required.
 - 3. Non-permitted providers that wish to provide BLS ambulance coverage of a special event within the County shall apply for temporary permit for special events coverage within the county.
 - 4. Non-permitted providers shall be required to pay a permit fee for the rights of County authorization of \$20.00/day to a maximum fee of \$100.00/year from date of issue. The fee may be waived if the service is an event that is considered to benefit Public Health, Welfare and/or Safety (at the discretion of the Medical Director or designee).

- E. Duration of Special Event Authorization/Accreditation - For the purpose of providing ALS or BLS medical coverage for special events, authorization/accreditation shall be effective only for the date(s) of the event while at the event site.
- F. Medical Control – There shall be medical control provided for all special events by the authorized Base Hospital for the service area.

III. PROCEDURE FOR ALS COVERAGE.

- A. The permitted ALS Provider shall, if applicable:
 - 1. Make all necessary arrangements with the appropriate Base Hospital for medical control.
 - 2. Make all necessary arrangements with the authorized dispatch center and ambulance company servicing the operating area which encompasses the special event for dispatch, communication and patient transport.
 - 3. Make all necessary arrangements with air ambulance transport (if applicable).
 - 4. Orient all ALS personnel covering the special event to the requirements listed above for the local ALS provider.
- B. The non-permitted ALS Provider shall:
 - 1. Be a provider in good standing within a county in the State of California.
 - 2. Complete and submit an application to the Solano County EMS Agency for approval at least fifteen (15) working days prior to the event. The Agency shall ensure that provider meets all local authorization requirements and shall notify the appropriate permitted ambulance providers (if the service elects to provide only BLS services) as soon as the special event provider is approved.
 - 3. Make all necessary arrangements prior to the special event with a local Base Hospital to provide medical control and have an acceptable plan for Base Hospital communication and patient documentation.
 - 4. Make all necessary arrangements, prior to the event, with the locally authorized ambulance service(s), both ground and air (if applicable), for patient transport.
 - 5. Make all necessary arrangements with the local designated dispatch center for all communication requirements.
 - 6. Ensure that all ALS personnel providing care for said provider possess, at a minimum, temporary ALS accreditation within the county in which the event is taking place, and have been oriented regarding all arrangements made with the Base Hospital and local transporting ambulance service(s). All personnel shall limit their scope of practice to that for which they are licensed or certified.
 - 7. Agree to have available, on the grounds of the special event:

- a. If not providing an ambulance, all appropriate ALS equipment as required by the Solano County EMS Agency, OR
 - b. If providing an ambulance, or a mobile intensive care unit 9MICU), completely stocked according to the policies of the county in which it is based. This ambulance must also be currently licensed by the California Highway Patrol for operation in the county in which it is based pursuant to Article I, Section 2501 of the California Vehicle Code (a copy of this license must accompany the application).
8. Agree to maintain documentation utilizing the Prehospital Care Report (PCR) approved for use by Solano County EMS for all patient contacts pursuant to Sections 100168 (a,6A) and 100169(e) of the California Code of Regulations. Copies of the PCR for all ALS patients, all BLS patients that are transported to a hospital, and all patients who refuse treatment must be forwarded to the Base Hospital for routine medical review. On all patient transports, the patient medical record copy of the PCR must either accompany the patient to the hospital or be forwarded to the receiving hospital within twenty-four (24) hours following the event.
9. Pay application fees.
10. A bond using any format approved by the County Auditor equal to 150% of the highest fine for a performance failure and the cost for a communication link with the CMED to coordinate ground and air ambulance³ response to the area of special coverage.
- C. ALS Staffing – For each ALS team, at least one member must be an EMT-P. Any ALS personnel not currently accredited within Solano County may receive temporary accreditation to work for an authorized ambulance provider for the purpose of supplying medical coverage for special events on completion of the following requirements:
 1. Must possess current certification, in good standing, as an EMT-P in the State of California and possess local accreditation in a county within the State.
 2. Must complete, submit and have approved by the Solano County EMS Agency, an application for temporary accreditation.
 3. Must be oriented to the Base Hospital communication and patient transport arrangements for the special event.
 4. Pay temporary accreditation fees.
 5. Have knowledge of the best transport route to the closest appropriate receiving facility.
 6. Shall be assigned a “special event” unit name which utilizes the name of the special event during all communications.

IV. PROCEDURE FOR BLS COVERAGE

- A. The permitted BLS provider shall, if applicable;

1. Make all necessary arrangements with the appropriate Base Hospital for medical control.
 2. Make all necessary arrangements with the authorized dispatch center and ambulance company servicing the operating area which encompasses the special event for dispatch, communication and patient transport.
 3. Make all necessary arrangements with air ambulance transport (if applicable).
 4. Orient all BLS personnel covering the special event to the requirements listed above for the local ALS provider.
 5. Have, in writing, an action plan approved by the EMS Office, to obtain immediate ALS back-up when clinically appropriate.
- B. The non-permitted BLS provider shall:
1. Be a provider in good standing within a county in the State of California.
 2. Complete and submit an application to the Solano County EMS Agency for approval at least fifteen (15) working days prior to the event. The Agency shall ensure that provider meets all local authorization requirements and shall notify the local ambulance provider as soon as the special event provider is approved.
 3. Make all necessary arrangements prior to the special event with a local Base Hospital to provide medical control and have an acceptable plan for Base Hospital communication and patient documentation.
 4. Make all necessary arrangements, prior to the event, with the locally authorized ambulance service(s), both ground and air (if applicable), for patient transport.
 5. Make all necessary arrangements with the local designated dispatch center for all communication requirements.
 6. Ensure that all BLS personnel providing care for said provider possess current certification as EMT-1As, and have been oriented regarding all arrangements made with the Base Hospital and local transporting ambulance service(s).
 7. Agree to have available, on the grounds of the special event, all appropriate BLS equipment and staff as required by the California Highway Patrol for operation in the county in which it is based, pursuant to Article I, Section 2501 of the California Vehicle Code (a copy of this license must accompany the application).
 8. Agree to maintain documentation utilizing the Prehospital Care Report (PCR), approved for use within the County, of all patient contacts pursuant to Sections 10085(f) of the California Code of Regulations. Copies of the PCR for all BLS patients that are transported to a hospital, and all patients who refuse treatment must be forwarded to the Base Hospital for routine medical review. On all patient transports, the patient medical record copy of the PCR must

either accompany the patient to the hospital with the transporting ambulance, or be forwarded to the receiving hospital within twenty-four hours following the event.

9. Pay application fees.
10. A bond using any format approved by the County Auditor equal to 150% of the highest fine for a performance failure and the cost for a communication link with the CMED to coordinate ground and air ambulance response to the area of special coverage.

C. BLS Staffing – All BLS personnel (EMT-1A) shall:

1. Possess current certification, in good standing, as an EMT-1A in the State of California.
2. Be oriented to the Base Hospital communication and patient transport arrangements for the special event.
3. Have knowledge of the best transport route to the closest appropriate receiving facility.
4. Be oriented regarding the appropriate method of contacting the designated dispatch center to coordinate the dispatch of the ALS ambulance company servicing the operating area encompassing the special event.
5. Pay temporary accreditation fee.
6. Shall be assigned a “special event” unit name which utilizes the name of the special event during all communications.

V. MEDICAL CONTROL

- A. A Base Hospital within Solano County that is contacted by an authorized provider for the purpose of providing ALS or BLS medical control for a special event shall provide on-line and off-line medical control for the provider for said special event and will conduct a medical review of all ambulance transports and a medical review of patient contacts which occur during the event.
- B. The EMS Agency shall notify the Base Hospital upon the approval of the provider’s application and provide the Base Hospital with a list of the EMT-Ps accredited for the ALS event(s).

VI. TRANSPORTATION

- A. Ambulance service(s) providing ambulance coverage to the zone in which the special event is being held shall provide transport services for all patients requiring ambulance transport from the special event except under the following conditions:
 1. The ambulance service does not have an ambulance immediately available;

2. If the ETA of the responding ambulance is greater than the transport time to the closest appropriate facility;
 3. If, in the Base Hospital's opinion, a patient's condition warrants immediate transport.
- B. The Base Hospital may order transport via the ALS or BLS ambulance and/or EMS aircraft temporarily authorized for the special event coverage.
1. Prior to transferring any patient to another provider, the patient's condition and delay caused by transferring care should be considered. Such transfer of a patient shall require the order of the Base Hospital.
 2. Rendezvous Point – In the event that the Base Hospital should order transport by the special event provider, and it is in the best interest of the patient, if at all possible or practical a rendezvous point should be arranged to transfer transportation of the patient to an authorized transport provider to enable the ambulance covering the special event to return for coverage.

A rendezvous should especially be considered by the Base Hospital if the transporting special events ambulance is a BLS ambulance or if the transporting special events ambulance is an ALS ambulance and the transport time is geographically isolated and/or outside ambulance response to the special event poses a risk to Public Health/Welfare and/or Safety.
